

WEINSTEIN MANAGEMENT CO.,
INC. AND WMCI DALLAS X, LLC,
Plaintiffs and Counter-Defendants,

v.

KATHRYN COPELAND,
Defendant and Counter-Plaintiff.

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IN THE DISTRICT COURT OF

TARRANT COUNTY, TEXAS

141ST JUDICIAL DISTRICT

**Reply to Plaintiffs' Response to Defendant's Verified Motion for Limited Reconsideration
of October 29, 2025 Order & Temporary Mandatory Injunction Request Relief
and Motion to Dismiss Frivolous and Malicious Claims**

I. INTRODUCTION

Plaintiffs' Response rests on two incorrect premises:

- (1) that this Court should ignore Plaintiffs' **own 93-day pattern of providing temporary housing "until remediation is completed,"** and
- (2) that lease expiration extinguishes their obligations under the **Fair Housing Act (FHA)** and **Americans with Disabilities Act (ADA)**.

Both are legally wrong.

This Motion does not seek damages. It seeks restoration of the *status quo Plaintiffs themselves created*: ADA/FHA-mandated temporary housing during ongoing remediation of an apartment their own licensed professionals deemed **"unfit for human occupancy."**

The emergency arises from Plaintiffs' **November 3, 2025** repudiation of that accommodation—even while admitting in sworn filings (October 20) that remediation continues. This repudiation, not the August 15 promise, is the triggering new evidence warranting reconsideration and temporary mandatory relief.

Homelessness, disability deterioration, and denial of accommodations constitute **irreparable harm as a matter of law** under the FHA and ADA. Plaintiffs cite no authority allowing a landlord to avoid federal civil-rights duties simply because it chooses not to renew a lease.

PRELIMINARY NOTE REGARDING PLAINTIFFS' NOVEMBER 24 RESPONSE

Plaintiffs' newly filed Response does not cure the defects in their prior arguments and, in several respects, reinforces the basis for reconsideration and mandatory relief. The Response continues to mischaracterize the governing federal law, ignores the controlling fact that Plaintiffs themselves provided 93 days of ADA/FHA housing "until remediation is completed," and now admits additional facts demonstrating that (1) remediation is still not complete, (2) the September 8 licensed protocol required pack-out, cleaning, negative pressure, and clearance, and (3) Plaintiffs withdrew accommodations after they were already granted.

Most critically, the Response does not address the central issue: **Plaintiffs' November 3, 2025 withdrawal of the accommodation they performed for 93 days is an independently actionable FHA violation and constitutes ongoing discrimination and retaliation as a matter of law.** Federal courts have repeatedly held that trial courts have **no discretion** to permit ongoing FHA violations to continue pending litigation. *See Woods-Drake v. Lundy*, 667 F.2d 1198 (5th Cir. 1982); *U.S. v. Wagner*, 930 F. Supp. 1148 (N.D. Tex. 1996); *Lincoln v. Case*, 340 F.3d 283 (5th Cir. 2003). Nothing in Plaintiffs' November 24 filing addresses — much less refutes — these controlling authorities or the statutory requirement that accommodations cannot be withdrawn once granted.

Accordingly, the newly filed Response only underscores the urgency of reinstating the accommodation Plaintiffs provided for 93 days and preventing ongoing irreparable harm.

II. GOVERNING LAW

A. Fair Housing Act

A “discriminatory housing practice” includes discrimination, failure to accommodate, and retaliation. 42 U.S.C. §§ 3602, 3604(f), 3617.

Federal courts routinely grant injunctive relief where ongoing FHA violations are shown. 42 U.S.C. § 3613(c)(1). Ongoing discrimination constitutes **irreparable harm as a matter of law**.

The Fifth Circuit authorizes punitive damages where a landlord acts with willful disregard of FHA rights—including ignoring known health risks or retaliating after a tenant invokes FHA protections. *See U.S. v. Wagner*, 930 F. Supp. 1148 (N.D. Tex. 1996); *Lincoln v. Case*, 340 F.3d 283 (5th Cir. 2003); *Woods-Drake v. Lundy*, 667 F.2d 1198 (5th Cir. 1982).

B. Texas Law: Habitability & Constructive Eviction

Texas law recognizes constructive eviction where the premises are unsafe or “unfit for human occupancy” and the tenant must vacate. *See Hamaker v. Newman*, 641 S.W.3d 867 (Tex. App.—Fort Worth 2022, pet. denied); *Charalambous v. Jean Lafitte Corp.*, 652 S.W.2d 521 (Tex. App.—El Paso 1983, writ ref’d n.r.e.).

A landlord may not collect rent or leverage the judicial process when its own actions rendered the premises uninhabitable.

C. Anti-Retaliation: FHA § 3617 & Texas Property Code § 92.331

Within six months of protected activity, a landlord may not retaliate through eviction, decreased services, or interference with rights. Texas Prop. Code § 92.331.

Federal courts applying Texas law recognize retaliation where a landlord takes adverse action shortly after a tenant requests an accommodation. *Chavez v. Aber*, 122 F. Supp. 3d 581 (W.D. Tex. 2015).

D. Court's Duty to Preserve Status Quo and Prevent Illegality

Courts must preserve the last peaceable, non-violating status quo and prevent ongoing illegality. *In re Newton*, 146 S.W.3d 648 (Tex. 2004).

Here, the status quo is the **93-day ADA/FHA accommodation Plaintiffs performed**—not the lease term.

III. PLAINTIFFS HAD ACTUAL NOTICE OF DEFENDANT'S DISABILITIES MONTHS BEFORE THE INJUNCTION HEARING

Plaintiffs' assertion that Defendant "failed to document" her disabilities is contradicted by their own records.

Key Disability Notices (All Before October 16 Hearing)

- **March 5, 2025:** Written disclosure of MCAS, POTS, hEDS, long COVID, severe environmental reactivity (Ex. A).
- **June 18, 2025:** BioTex declares unit "unfit for human occupancy" (Ex. 13 to 9/29 Emergency Motion).
- **June 30, 2025:** Formal FHA/ADA accommodation request citing 42 U.S.C. § 3604(f) (Ex. B).
- **September 8, 2025:** Dr. Isabel Huang documents severe neurological, immunological, and vascular disability; 18 hours/day bedridden; 30-lb weight loss; urgent need for relocation (Ex. C).
- **September 29, 2025:** Filed into the record with Emergency Motion.
- **October 23, 2025:** Re-urged ADA/FHA accommodation request in Motion for Temporary Injunction.

These records satisfy Plaintiffs' claimed requirement that disability be "explicitly delineated by a physician." Response at 26.

Plaintiffs Possessed This Documentation Before the TI Hearing

Plaintiffs possessed all of this medical documentation when they appeared at the October 16 temporary-injunction hearing. Dr. Isabel Huang's September 8 physician statement—filed as Exhibit 12 to Defendant's September 29 Emergency Motion—explicitly documented Defendant's disabling conditions, functional limitations (“unable to perform sustained work activity”), mold-related causation, and the “urgent need for relocation” as a medical accommodation. (Ex. 12, p. 104.)

This same physician statement satisfies Plaintiffs' own asserted requirement that accommodations “must be explicitly delineated by a physician.” Response at 26 (Requested Finding No. 6). Plaintiffs' continued claim that Defendant “failed to document” her disabilities is thus directly contradicted by documents they already possessed weeks before the hearing.

Legal Effect

Under 24 C.F.R. § 100.204, once a housing provider has notice of a disability, the obligation to engage in a good-faith interactive process is immediately triggered. Defendant provided such notice repeatedly—first in March, then formally in June with explicit citation to FHA § 3604(f)(3)(B), and again in September with detailed physician documentation. Plaintiffs never engaged in any interactive process. Instead, they refused accommodations, provided medically inappropriate lodging, ignored specific accessibility requests, and ultimately terminated substitute housing while admitting remediation was incomplete. This conduct constitutes discrimination and failure to accommodate under the FHA.

Knowing Conduct

On September 16, Defendant cited FHA § 3604(f)(3)(B) and *Schwarz v. City of Treasure Island* in writing, explaining Plaintiffs' accommodation duties. On September 18, Defendant again

cited ADA communication-accommodation obligations. Plaintiffs therefore acted with full knowledge of their statutory obligations when they refused to engage in the interactive process and then pursued retaliatory actions.

III-A. The Record Independently Establishes Disability Under FHA and ADA

The record contains ample evidence satisfying the FHA definition of “handicap,” 42 U.S.C. § 3602(h), and the ADA definition of “disability,” 42 U.S.C. § 12102(1). Medical records from Dr. Huang, Dr. Johnson, and Dr. Conte document physical impairments (neurological injury, immunological dysfunction, mycotoxin toxicity, vision loss) that substantially limit major life activities including seeing, breathing, standing, concentrating, and working. Expert reports further connect these impairments to environmental exposure in Unit 2145. Plaintiffs’ own conduct—providing 93 days of hotel accommodations and repeatedly directing Defendant not to return—also evidences an acknowledgment of disability and need for accommodation. This is more than sufficient to establish disability for purposes of FHA/ADA relief.

Plaintiffs' November 24 Response still does not dispute that they possessed Dr. Huang's September 8 physician statement documenting disability before the October 16 hearing. Their continued refusal to engage in any interactive process is itself an FHA violation.

IV. PLAINTIFFS FAILED TO ENGAGE IN THE INTERACTIVE PROCESS

The FHA requires landlords to engage in a flexible, interactive dialogue to identify reasonable accommodations.

Plaintiffs instead:

- **June 30:** Issued a blanket refusal (“We are not able to accommodate...”).
- **July–August:** Provided medically inappropriate, non-ADA-compliant lodging.
- **September 16:** Ignored explicit ADA-compliant housing requests.
- **September 19:** Filed this lawsuit **three days** after receiving a detailed FHA/ADA request.
- **Never** proposed alternatives.
- **Never** engaged in any meaningful dialogue.

No interactive process occurred. This is itself an FHA violation.

V. PLAINTIFFS’ RETALIATION IS CLEAR AND UNDISPUTED

The timing speaks for itself:

- **June 27:** FHA/ADA accommodation request.
- **August 15:** Insurer promises housing “**until remediation is completed.**”
- **August 26:** Lease non-renewal—**11 days later.**
- **October 26:** Termination of hotel housing.
- **November 3:** Repudiation of the accommodation while admitting remediation continues.

Issuing adverse actions within days of an accommodation request is **prima facie retaliation** under FHA § 3617 and Texas Prop. Code § 92.331.

VI. THE NOVEMBER 3 REPUDIATION IS THE TRIGGERING NEW EVIDENCE

The breach occurred **November 3**, when Plaintiffs terminated housing despite admitting remediation was incomplete.

Plaintiffs' argument that the August 15 email is "80 days old" ignores that **repudiation**, not promise, controls timeliness.

Defendant acted diligently:

- **Oct 27:** Housing terminated.
- **Nov 1:** Filed JP Repair & Remedy.
- **Nov 18:** JP dismissal (procedural; reinstated).
- **Nov 19:** Emergency Motion filed in this Court.

This is diligence—not delay.

VII. LEASE EXPIRATION DOES NOT EXTINGUISH FEDERAL CIVIL-RIGHTS OBLIGATIONS

A. Constructive Eviction Occurred Long Before October 26

- **June 18:** Unit declared "unfit for human occupancy."
- **July 14:** Defendant displaced into hotels.
- **Sept 22–26:** All belongings removed without consent.
- Plaintiffs repeatedly stated Defendant could not return.

These facts establish constructive eviction under *Hamaker*.

B. FHA/ADA Duties Run to the Person, Not the Lease

FHA/ADA obligations attach to a person who invokes rights—not to the existence of a lease. Retaliation can occur **after tenancy ends**.

Plaintiffs cite **no contrary authority**.

C. The “Lease End Date” Cannot Erase Federal Civil Rights Violations

Federal civil-rights obligations do not terminate because a landlord chooses not to renew a lease. Plaintiffs’ contention that the November 30, 2025 lease expiration extinguishes their FHA/ADA duties finds no support in statute, regulation, or case law.

1. FHA/ADA duties attach to the person, not the tenancy

Under both the FHA and ADA, a housing provider’s obligations arise when a disabled individual requests an accommodation. **42 U.S.C. §§ 3604(f)(3)(B), 3617; 24 C.F.R. § 100.204.**

These duties continue until the discriminatory conduct stops, regardless of lease status. Courts routinely hold that landlords remain liable for discrimination or retaliation **even after a tenancy ends.**

2. Retaliation is actionable post-tenancy

Section 3617 prohibits coercion, intimidation, threats, interference, or retaliation “in the exercise or enjoyment of” FHA rights. This includes:

- withdrawing an accommodation already granted,
- terminating housing in response to protected activity,
- or taking adverse actions after a tenant invokes rights.

The Fifth Circuit and Texas district courts hold that **retaliation claims survive the end of occupancy** because the violation is the retaliatory act—not the lease. *See Woods-Drake v. Lundy*, 667 F.2d 1198 (5th Cir. 1982); *Chavez v. Aber*, 122 F. Supp. 3d 581 (W.D. Tex. 2015).

3. Plaintiffs’ 93-day course of conduct created a statutory status quo

For 93 days, Plaintiffs:

- paid hotel housing,
- represented housing would continue “until remediation completed,”
- repeatedly admitted “remediation continues,” and
- instructed Defendant not to return to the unit.

Under *In re Newton*, 146 S.W.3d 648 (Tex. 2004), the Court must preserve the “last peaceable, non-violating status quo”—which is the accommodation Plaintiffs provided, not the lease expiration.

4. Plaintiffs cannot use lease expiration as a shield to evade federal civil-rights liability.

Their theory would allow a landlord to:

1. violate FHA rights,
2. retaliate for protected activity,
3. refuse accommodations,
4. simply not renew a lease, and
5. claim the violations evaporate.

That is not the law. Lease timing cannot erase statutory violations.

5. The November 3 repudiation—while admitting remediation is incomplete—is the continuing violation.

The controlling event is **not** the lease expiration. It is the **November 3 withdrawal of an already-granted accommodation** while Plaintiffs admitted remediation was still incomplete.

Withdrawing an accommodation is itself:

- discrimination under § 3604(f)(3)(B), and
- retaliation under § 3617.

Ongoing discrimination = irreparable harm as a matter of law.

42 U.S.C. § 3613(c)(1); *Woods-Drake*; *Wagner*; *Lincoln*.

Bottom line:

The “lease end date” argument is a legal fiction. Federal civil-rights obligations **do not vanish** because a landlord chooses not to renew a lease. Plaintiffs provided accommodation for 93 days, relied on it in court, then abruptly repudiated it while remediation remained incomplete.

Federal civil-rights duties continue regardless of lease expiration, and **the Court has no discretion to allow discrimination or retaliation to continue.**

VIII. PLAINTIFFS' OWN DOCUMENTS ESTABLISH THE STATUS QUO THEY NOW DISAVOW

For **93 days**, Plaintiffs:

- Paid \$22,213 for hotel displacement
- Extended housing repeatedly
- Directed Defendant not to return
- Admitted “remediation continues” (Oct 20)
- Possessed a September 8 licensed protocol requiring pack-out, cleaning, negative pressure, and clearance

These acts constitute the operative status quo. Plaintiffs offer no explanation of how these admissions are compatible with their new “no housing owed” theory.

Status quo = the accommodation Plaintiffs provided.

IX. PLAINTIFFS' CONSTRUCTIVE-EVICTION ARGUMENT FAILS

Plaintiffs cite commercial cases where tenants continued operating businesses. Defendant did not continue using the premises; she was medically displaced, placed in hotels, and expressly prohibited from returning.

Brief returns to retrieve medication or document conditions do not negate constructive eviction. *Hamaker* controls.

IX-A. Newly Discovered Contradictions Undermine Plaintiffs' Credibility

Since the October 16 hearing, Defendant obtained Plaintiffs' internal emails, their September 8 licensed remediation protocol, and insurance communications showing multiple material contradictions between what Plaintiffs told the Court and what their own documents show. These contradictions concern (1) concealment of the September 8 protocol, (2) false statements about “unregulated” contents cleaning, and (3) reversal of the August 15 housing

commitment while admitting remediation was still ongoing. These newly discovered facts go directly to credibility, demonstrate ongoing violations of federal law, and strongly support temporary mandatory relief. A summary of these contradictions is attached in supporting exhibits.

X. TEMPORARY MANDATORY RELIEF IS WARRANTED

1. Likelihood of Success

Plaintiffs' own documents prove:

- failure to accommodate
- retaliation
- constructive eviction
- continuing uninhabitability

2. Irreparable Harm

Homelessness, worsening disability, and civil-rights violations constitute irreparable harm as a matter of law.

3. Balance of Equities

Plaintiffs already provided hotel housing for 93 days. Restoring this imposes minimal burden and simply reinstates their own accommodation.

4. Public Interest

Federal law strongly favors preventing discrimination and retaliation during litigation.

XI. CONCLUSION

This Motion seeks to restore the accommodation Plaintiffs themselves provided for 93 days: temporary ADA/FHA-compliant housing during ongoing remediation. The **November 3 repudiation** created the present emergency while Plaintiffs simultaneously admitted “remediation continues.”

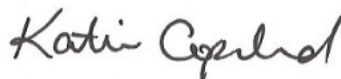
Plaintiffs cite no FHA or ADA authority supporting the notion that federal civil-rights obligations evaporate when a landlord elects not to renew a lease. The status quo Plaintiffs established—not the lease term—is what the Court must preserve.

XII. PRAYER

Defendant respectfully requests that the Court:

1. **Grant the Emergency Motion for Limited Reconsideration** (or the October 23, 2025 filed Defendant's Emergency Motion to Vacate Temporary Injunction);
2. **Enter a temporary mandatory injunction reinstating temporary housing**, consistent with the course of accommodation Plaintiffs established or in the alternative, award interim housing costs;
3. **Preserve all property and evidence pending trial;**
4. Abate/stay rent collection and the November 6 Notice to Vacate pending resolution of constructive eviction and FHA claims; Enter a protective order limiting third-party discovery to prevent ongoing retaliation; and
5. Grant all additional relief to which Defendant is entitled.

Respectfully submitted,



KATHRYN COPELAND

State Bar No. 24086056

405 Crawford St. #2145

Fort Worth, Texas 76104

Tel: (817) 789-8498

Katie@CopelandLawTexas.com

Defendant and Counter-Plaintiff, Attorney Pro Se

**UNSWORN DECLARATION PURSUANT TO
TEXAS CIVIL PRACTICE & REMEDIES CODE 132**

“My name is Kathryn Marie Copeland, my date of birth is June 28, 1985 and my current address is 405 Crawford Street #2145, Fort Worth, TX 76104 in the United States of America. I declare under penalty of perjury that the foregoing is true and correct. Executed in Tarrant County, State of Texas, on the 23rd day of November 2025.”



KATHRYN COPELAND

CERTIFICATE OF SERVICE

I hereby certify that a true and correct copy of this motion was served on November 24, 2025 via electronic service to:

Glynis L. Zavarelli
Wentz & Zavarelli, LLP
gzavarelli@wandzlaw.com

Michelle S. Sortor
Wentz & Zavarelli, LLP
msortor@wandzlaw.com



KATHRYN COPELAND

Exhibit A

March 5, 2025

Subject: Urgent: Mold Issue & Health Concerns

Date: Wednesday, March 5, 2025 at 12:52:24 PM Central Standard Time

From: Katie Copeland

To: Bowery Southside

Hello, I never realized mold was so serious. I just asked ChatGPT about the mold and my unique constellation of diseases. It told me this was urgent and suggested I write the following:

I am reaching out to report a concerning mold issue in my apartment. I have noticed mold growing in the corner of my entryway, which raises concerns that there may be mold in other areas as well. Given my medical conditions, this is an urgent issue that could severely impact my health.

I have several chronic illnesses, including mast cell activation syndrome (MCAS), celiac disease, and long COVID, among others, which make me extremely sensitive to environmental triggers like mold. Mold exposure can cause severe allergic reactions, respiratory issues, immune system dysfunction, and increased inflammation, all of which could seriously worsen my condition.

Because of these health risks, I need an inspection as soon as possible to determine the extent of the mold issue and whether remediation is necessary. If testing confirms a mold problem, I will need immediate intervention to ensure my apartment remains a safe living environment.

Please let me know how soon we can schedule an inspection and discuss next steps. I appreciate your prompt attention to this matter, as it directly affects my ability to safely remain in my home.

Separate question: do you know anyone who does dog walking in our complex or nearby? I had a medical procedure done today and it would be great if I could pay someone to walk my dogs since I am not able to move much.

Best,

Katie Copeland
817-789-8498

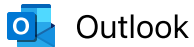






Exhibit B

June 30, 2025



Re: ADA/FHA Accommodation Request and Notice of Urgent Health Risk – Unit #2145

From Copeland, Katie <K.M.COPELAND@tcu.edu>

Date Mon 6/30/2025 8:50 PM

To The Bowery - Kodi Walker <kowalker@weinsteinproperties.com>; LBenaicha@weinsteinproperties.com <lbenaicha@weinsteinproperties.com>

Dear Ms. Walker and Legal Counsel:

This message serves as formal notice that I have filed complaints with the following agencies regarding Weinstein Properties' ongoing violations of Texas and federal law:

1. **Texas Department of Licensing and Regulation (TDLR)** – Complaint #0000018107 for unlicensed mold assessment and remediation in violation of *Texas Occupations Code § 1958.101*
2. **U.S. Department of Housing and Urban Development (HUD)** – Complaint for denial of reasonable accommodation under the *Fair Housing Act* and *Americans with Disabilities Act (ADA)*

Your team's attempt to perform in-house assessment and remediation on a unit declared **“unfit for human occupancy”** by licensed Mold Assessor Kyle Reist (License #MAC1742), with mold-affected areas exceeding 25 contiguous square feet, is unlawful under Texas law.

“A person may not engage in mold assessment or mold remediation unless licensed to do so.” – *Tex. Occ. Code § 1958.101(a)*

To be clear, my prior consent to entry on June 27, 2025, was **limited to visual observation only**. At no point did I authorize an assessment, demolition, unlicensed remediation, or baseboard removal. Your team's actions have worsened my documented medical conditions and further contaminated personal property belonging to me, my children, and our service animals.

I do not consent to any further entry for unlicensed remediation or inspection. Any such attempt will be documented and added to the complaints already filed with TDLR and HUD.

I continue to request **reasonable accommodation** under the ADA and Fair Housing Act based on documented disability and medical necessity. A licensed environmental professional has confirmed my unit is **unsafe to occupy**, and my treating providers support immediate removal from the exposure environment. I have proposed a 14-day temporary relocation to The Nobleman Hotel (availability confirmed), with boarding for my service animals at Doggie Diggs next door.

Despite clear evidence of health risk and legal obligation, you have declined to facilitate temporary relocation. The denial of this medically necessary request creates clear exposure under the ADA, FHA, and *Texas Health & Safety Code*.

I request the following by **July 1, 2025 at 4:00 PM Central Standard Time**:

1. Written confirmation that **no unlicensed remediation** will be attempted;
2. Approval of the requested **reasonable accommodation** or a legally compliant alternative;
3. The **name and contact information** of Weinstein Properties' **insurance carrier** for this matter;
4. Contact details for **Weinstein Properties' legal counsel**.

I have acted in good faith and remain open to lawful, cooperative resolution. However, I will continue to pursue all available remedies under state and federal law if necessary.

I would like to know how you intend to offer this—especially if there are no comparable units available. Will you be assisting with relocation logistics? I would like to consider all options, but in the meantime, I stand firm in my position that immediate relocation is medically necessary and legally protected.

I reject your characterization that the unit is safe to occupy. This conclusion directly contradicts the licensed mold assessor's written findings, my documented medical conditions, and applicable state and federal law.

Sincerely,
Katie Copeland
(817) 789-8498
[REDACTED]

From: The Bowery - Kodi Walker <kowalker@weinsteinproperties.com>
Date: Monday, June 30, 2025 at 5:42 PM
To: Copeland, Katie <K.M.COPELAND@tcu.edu>
Subject: RE: ADA/FHA Accommodation Request and Notice of Urgent Health Risk – Unit #2145

[EXTERNAL EMAIL WARNING] DO NOT CLICK LINKS or open attachments unless you recognize the sender and know the content is safe.

Hi Katie –

I want to start by emphasizing that your health and safety are always our top priority. We take this matter very seriously, and I want to personally assure you that our team is trained to follow all EPA guidelines when performing repairs of this nature. The work we're prepared to carry out is aligned with these standards to ensure everything is handled safely and effectively.

I want to confirm that there is no active leak detected in your home, and all necessary repairs have already been completed. Our team has conducted a thorough assessment, and moisture readings

confirm that the area is dry. With no signs of water present, there is no risk of continued mold growth at this time.

City's Codes and Compliance department recently stopped by and informed us that they had visited your home. While we have not yet received an official report, they have requested that the necessary repairs be completed timely. With that in mind, we would like to schedule the repair work for tomorrow to resolve this quickly and minimize any disruption to your routine. This includes, but is not limited to, the replacement of baseboards and effected areas of sheetrock. The work is limited in scope, non-invasive, and can be completed within a day with minimal to no disruption to your daily routine. However, if you would prefer for us to complete the work while you are away from the home for your own peace of mind, we are more than happy to coordinate a time that works better for you.

As mentioned, the scope of the work is limited, non-invasive, and will be completed within a day. Based on our assessment, there is no need for you to vacate the home or arrange for hotel accommodations. At this time, we are not able to accommodate requests for relocation or hotel reimbursement. However, if you feel that remaining in the home during this process is not comfortable for you, we completely understand and want to support you. We're happy to offer the option to transfer to another unit or to end your lease early without penalty.

Since you reported this issue, it is important that we demonstrate our responsiveness and complete the necessary repairs.

Please feel free to reach out with any questions or concerns!

Thank you,



Kodi Walker | Property Manager

The Bowery at Southside Apartments
220 East Broadway Ave. | Fort Worth, TX 76104
Call or Text **833.887.1948**
[WEBSITE](#) | [RESIDENT SERVICES](#)

From: Copeland, Katie <K.M.COPELAND@tcu.edu>
Sent: Monday, June 30, 2025 2:09 PM
To: The Bowery - Kodi Walker <kowalker@weinsteinproperties.com>
Cc: Lile Benaicha <lbenacha@weinsteinproperties.com>; wp@weinsteinproperties.com
Subject: ADA/FHA Accommodation Request and Notice of Urgent Health Risk – Unit #2145
Importance: High

Caution: This is an EXTERNAL EMAIL - Be cautious of clicking links and opening attachments.

If you suspect that this is a phishing email, report this to IT by entering a computer help ticket.

Dear Ms. Walker,

This message constitutes a formal request for reasonable accommodation under the Americans with Disabilities Act (ADA), the Fair Housing Act, and related Texas laws.

I understand from Fort Worth Code Compliance that you claim I am not cooperating with remediation. That statement is demonstrably false. I have

repeatedly offered access and expressed willingness to allow remediation, as documented in our ongoing correspondence. My sole concern has been ensuring the work is conducted **legally, safely, and by licensed professionals**, as required under Texas law.

As you are aware, I have multiple serious health conditions—including immunological, neurological, and vascular disabilities—that substantially impair major life activities and render me particularly vulnerable to environmental triggers such as mold toxins. These are documented by my treating physicians and home health providers.

Despite repeated notice—including my March 5 email—you and your team assured me the issue had been addressed. However, a licensed mold assessor has now confirmed the presence of **Stachybotrys, Chaetomium, and extensive moisture damage**, concluding that my unit is “**unfit for human occupancy.**” My symptoms—vision loss, chronic inflammation, nosebleeds, respiratory issues, cognitive dysfunction, severe fatigue, and burning eyes—have worsened significantly since your staff began removing baseboards and “inspecting” the unit without containment or proper licensure.

(See Texas Occupations Code Sec. 1958.101(a): A person may not engage in (1) mold assessment or (2) mold remediation unless licensed to do so.)

I am also attaching the **State Farm denial letter**, which confirms that **displacement costs are not covered**, contrary to your suggestion that I consult my renter’s insurance.

Reasonable Accommodation Request:

Under federal and state disability laws, I am requesting:

1. **Temporary relocation for at least 14 days** to The Nobleman Hotel (availability confirmed)
2. **Boarding for my two service animals** at Doggie Diggs next door during this time (\$85/night)
3. **Licensed mold remediation** in compliance with Texas Occupations Code Sec. 1958.101

This temporary relocation is a **reasonable accommodation directly tied to my disability**, and is **medically necessary** based on both physician input and the professional mold report. It would also allow your team full access for remediation without exposing me to further medical harm.

I am open to other creative solutions—hotel, extended stay, or temporary alternative unit—but I **cannot safely remain** in an environment now confirmed

to be hazardous to my health.

⚠ **Please confirm by 6:00 PM today that:**

1. This accommodation request is approved
2. No further demolition or unlicensed work will occur
3. Arrangements for The Nobleman and Doggie Diggs are being made immediately

I remain willing to fully cooperate with remediation—**so long as it is legal, licensed, and does not further endanger my health.** I hope this matter can be resolved cooperatively, without need for additional regulatory enforcement or litigation.

Sincerely,
Kathryn Copeland
405 Crawford Street, Apt. 2145
Fort Worth, TX 76104
(817) 789-8498

Attachments:

- BioTex Mold Report (6/18/25)
- State Farm Denial Letter
- March 5 Mold Notice Email
- Texas Occupations Code Sec. 1958



From: Copeland, Katie <K.M.COPELAND@tcu.edu>

Date: Friday, June 27, 2025 at 5:29 PM

To: The Bowery - Kodi Walker <kowalker@weinsteinproperties.com>

Subject: Re: URGENT HEALTH & SAFETY ISSUE – Formal Notice & Request for Accommodations – The Bowery at Southside Unit #2145

Ms. Walker,

Thank you for your response regarding the mold in my apartment. I'm writing to clarify several critical concerns before any remediation begins.

Exhibit C

**Pages 83-113 (Ex. 12) of
Defendant's Emergency Application for
Preservation Order and Temporary Relief filed
September 29, 2025**

Consists of medical records that detail Ms.
Copeland's functional limitations and
disability status

Exhibit 12

Encounter Review - Exam - Initial

Monday, July 7, 2025 at 11:02 AM, by Alfred R. Johnson D.O.
997 Hampshire LN, Richardson, TX 75080-8105 • (972) 479-0400



Kathryn M. Copeland
405 Crawford St #2145
Fort Worth, TX 76104

Patient ID: 91408794
Incident: Office Visit

Date of Birth: [REDACTED]
Age: 40

Gender: Female
Marital Status: Divorced

Employment Status: Employed

CHIEF COMPLAINT

Constitutional

- CC: mold exposure/legal case
Realtor: 5/5 present (6/2025)
Address: 405 Crawford St, Apt 2145, Fort Worth, TX 76104
Moved in Oct, 2023 and still there.
Flooding in bedroom - floor wet and maintenance supposedly fixed a backed-up HVAC leak - Sept, 2024. Mold visible by front door (first reported in Oct, 24 and then came back in March, 25 even tho supposedly remediated in Oct and then mold returned in early June).
Visible mold in front of HVAC on baseboard - not where the first leak was - water damage was painted over before she moved in.
Mold AQL - Elevated Aspergillus/Penicillium/Stachy on Air quality - 6/19/25
Guest Bedroom - Asp (5000) Chaetomium (14, 000) Stachy (1500) Total fungi =20700
99% of mold was of toxic variety in guest bedroom and wall cavity
- Sxs: chronic fatigue (18 hrs laying down in a day), vision changes, brain fog (since Covid) and worsened, joint pain (hypermobile EDS), muscle pain, nerve pain, generalized weakness, skin rashes on face, sinus/ear issues, nose congestion, diarrhea/constipation.
- Occupation: was a lawyer until Dec., 2022. Then doing paralegal work until she got Covid and then stopped working.
- Covid - Dec, 2022 - mild case - 3 weeks getting over it. Have vaccine.
-

HISTORY OF PRESENT ILLNESS

Constitutional

- The patient presents with CC: mold exposure/legal case

Realttime: 5/5 present (6/2025)

Address: 405 Crawford St, Apt 2145, Forth Worth, TX 76104

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◦

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EXAMINATION

Constitutional

Height

- Height 63 inches.

Weight

- Weight 124.0 pounds.

Vitals

- BP 104/67 mmHg was taken sitting using the left arm.
- Pulse (regular) 88 beats per minute.

Ears, Nose, Mouth, and Throat

- Inspection and assessment of ears, nose, mouth, and throat, unless otherwise noted, reveals the following. Otoscopic exam of tympanic membranes and auditory canals shows no perforation, lesions, or infection. External inspection of the ears and nose reveals normal external appearance without lesions, scars or masses. Inspection of internal nose reveals normal appearing mucosa, with straight septum, turbinates of normal color and shape and no active bleeding. Inspection of mouth reveals normal color lips, and dentition without significant caries or gum disease. Examination of the oropharynx reveals pink, moist mucosa and midline tongue and no lesions or redness of the posterior pharynx, palate, or tonsils. Inspection of the pharyngeal walls and pyriform sinuses shows normal color and appearance, no lesions or masses, and clear pyriform sinuses.

Respiratory

- Examination of the chest and respiratory system, unless otherwise indicated, reveals the following. Inspection of the chest reveals a symmetrical chest with normal expansion. Assessment of respiratory effort reveals no intercostal retractions with normal diaphragmatic movement. Auscultation of lungs reveals normal breath sounds without rales, rubs, or rhonchi.

Cardiovascular

- Sounds: Rapid regular rate.

Normal

- Unless otherwise noted, palpation and auscultation of the heart appeared normal. Examination of the carotid arteries, abdominal aorta, femoral arteries, and pedal pulses revealed findings within normal limits. No pedal edema was seen. No pericardial friction

rub was heard.

Gastrointestinal

Abdomen

- Examination of the abdomen: Slight tenderness midline.

Musculoskeletal

Normal

- Examination of the musculoskeletal system, unless otherwise noted, reveals normal findings.

Neurological

- Testing, palpation, and inspection of the neurological system, unless otherwise noted, reveals the following: Coordination and fine motor skills are in normal range. Estimate of mood and affect show no evidence of depression, excessive anxiety, or agitation. The patient is oriented to time, place and person.

Eyes

Ocular Motility

- An ocular motility inspection was conducted: Exophthalmos bil.

Neck

- Inspection and palpation of the neck, unless otherwise noted, reveals the following. Palpation of the thyroid reveals a normal gland without enlargement, tenderness, or masses.



PROBLEMS LIST

No chronic health problems have been listed.

ASSESSMENT AND PLAN

Assessment

- Mold toxicity - Ochra and Glio the highest - Allermatrix, skin testing for molds, start antioxidants.
After reviewing IAQ which shows very high toxic mold in the pat - imperative that pt move out ASAP
Check basic labs, lgs, saccharomyces
 - 40 hr HBOT - start 1.5 ATM for 60 mins and then increase to 2.0ATM for 60 mins up to twice daily or 90 mins once daily
 - Repeat urine mycotoxins after 10 hrs of HBOT
 - Chronic fatigue/brain fog - QEEG + CNS VS
 - Hashimoto's - check TFTs, cortisol.

Assessment / Plans

- Resources and instructions provided during visit.
- There was a discussion of the patient's laboratory values.
- The patient expressed understanding of the topics discussed and of any instructions or recommendations that were given. The patient voiced agreement.
- Plans for return: return in 2 weeks.

CHRONOGRAPH

DESCRIPTION	07/07/2025
Blood Pressure	104/67
Height	63
Pulse	88
Weight	124.0

CHRONOGRAPH - LAB DATA

DESCRIPTION	07/12/2025
Albumin	4.3
Alkaline Phosphatase	54
ALT (SGPT)	12
Amylase	58
AST (SGOT)	23
Baso (Absolute)	0.0
Basos	1
BUN	14
BUN/Creatinine Ratio	19
Calcium	9.2
Carbon Dioxide, Total	23
Chloride	103
Cholesterol, Total	 237
Cortisol	 36.0
Creatine Kinase, Total	91
Creatinine	0.75
eGFR	103
Eos	1
Eos (Absolute)	0.1
GGT	13
Globulin, Total	2.2
Glucose	84
HDL Cholesterol	79
Hematocrit	42.0
Hemoglobin	13.5
IgG, Subclass 1	 452
IgG, Subclass 2	 217
IgG, Subclass 3	53
IgG, Subclass 4	37
Immature Grans (Abs)	0.0
Immature Granulocytes	1
Immunoglobulin A, Qn, Serum	116
Immunoglobulin E, Total	15
Immunoglobulin G, Qn, Serum	889
Immunoglobulin M, Qn, Serum	151

LDH	180
LDL Chol Calc (NIH)	 129
Lymphs	47
Lymphs (Absolute)	2.7
Magnesium	1.8
MCH	29.2
MCHC	32.1
MCV	91
Monocytes	9
Monocytes(Absolute)	0.5
Neutrophils	41
Neutrophils (Absolute)	2.4
Phosphorus	4.3
Platelets	230
Potassium	4.6
Protein, Total	6.5
RBC	4.62
RDW	14.0
Saccharomyces cerevisiae, IgA	 38.3
Saccharomyces cerevisiae, IgG	 31.8
SARS-CoV-2 Spike Ab Dilution	8370
Sodium	142
T. Chol/HDL Ratio	3.0
T4,Free(Direct)	1.03
Triglycerides	 170
Triiodothyronine (T3), Free	2.4
TSH	2.870
WBC	5.7

LAB AND TREATMENT ORDERS

Date	Status	Order	Instructions
Johnson - LabCorp - Lab Orders			
07/07/2025	Awaiting Results	[001974] {T4 Free Direct Serum}	
07/07/2025	Awaiting Results	[320918] {CMP14+LP+4AC+Mg+Amy}	
07/07/2025	Awaiting Results	[005009] {CBC With Differential/Platelet}	
07/07/2025	Awaiting Results	[001792] {Immunoglobulin M, Qn, Serum}	
07/07/2025	Awaiting Results	[002170] {Immunoglobulin E, Total}	
07/07/2025	Awaiting Results	[001784] {Immunoglobulin A, Qn, Serum}	
07/07/2025	Awaiting Results	[123225] {Immunoglobulin G. Serum}	
07/07/2025	Awaiting Results	[004259] {TSH}	
07/07/2025	Awaiting Results	[004051] {Cortisol}	
07/07/2025	Awaiting Results	[164657] {Saccharomyces cerevisiae Panel}	
07/07/2025	Awaiting Results	[164090] {Covid-19 Titer}	
07/07/2025	Awaiting Results	[164068] {Covid-19 Nucleocapsid}	
07/07/2025	Awaiting Results	[010389] {Triiodothyronine, Free, Serum}	

07/07/2025	Awaiting Results	Hyperbaric Oxygen Treatment	40 hr HBOT - start @ 1.5 ATM for 60 mins and then increase to 2.0ATM for 60 mins up to twice daily or 90 mins once daily
			Repeat urine mycotoxins after 10 hrs of HBOT
07/07/2025	Awaiting Results	QEEG	QEEG + CNS VS Dx: mold toxicity, brain fog, chronic fatigue
07/07/2025	Awaiting Results	Glutathione 600mg daily, COQ10 - 200mg daily, ALA 300mg twice daily	
07/07/2025	Awaiting Results	(skin test) Stachy, Aspergillus, Penicillium and Chaetium	
07/07/2025	Awaiting Results	CommonWealth IgG - wheat dairy candida gluten soy bk and br yeast corn rice oat garlic onion black pepper egg cane (15 items)	
07/07/2025	Awaiting Results	CommonWealth IgG mold	four molds

CPT CODES

- 99203 - e/m new patient - detailed history, detailed exam, low complexity decision.

TREATMENT ORDERS

- Order from Johnson Medical Assoc., P.A.. CommonWealth IgG mold . CommonWealth IgG - wheat dairy candida gluten soy bk and br yeast corn rice oat garlic onion black pepper egg cane (15 items).
- Order from Johnson Medical Assoc., P.A.. (skin test) Stachy, Aspergillus, Penicillium and Chaetium .
- Order from Johnson Medical Assoc., P.A.. Glutathione 600mg daily, COQ10 - 200mg daily, ALA 300mg twice daily .
- Order from Johnson Medical Assoc., P.A.. QEEG.
- Order from Johnson Medical Assoc., P.A.. Hyperbaric Oxygen Treatment.
- Order from Johnson - LabCorp. General medical tests - [164068] {Covid-19 Nucleocapsid} and [164090] {Covid-19 Titer}. [010389] {Triiodothyronine, Free, Serum}. [001974] {T4 Free Direct Serum}. [004259] {TSH}. [123225] {Immunoglobulin G, Serum} [001784] {Immunoglobulin A, Qn, Serum}. [002170] {Immunoglobulin E, Total} [001792] {Immunoglobulin M, Qn, Serum}. [005009] {CBC With Differential/Platelet} [320918] {CMP14+LP+4AC+Mg+Amy}, [004051] {Cortisol}, and [164657] {Saccharomyces cerevisiae Panel}.

Order Requested By: Johnson Medical Associates	
Johnson D.O., Alfred R. 997 Hampshire Lane Richardson, TX 75080-8105 Phone: (972) 479-0400	
Provider	
License #:	0BF8525
Patient	
Name:	91408794 - Copeland, Kathryn M.
Phone Number:	(817) 789-8498
DOB:	06/28/1985
Address:	405 Crawford St #2145 Fort Worth, TX 76104
Service Facility	
Name:	Johnson Medical Assoc., P.A.
Phone:	(972) 479-0400
Address:	997 Hampshire LN Richardson, TX 75080-8105
Orders	
Diagnosis:	Z77.120, R53.82, G31.84, F90.9, U09.9, K21.9, G
LOINC:	
Date	Descriptions <i>Instructions</i>
07/07/2025	Glutathione 600mg daily, COQ10 - 200mg daily, ALA 300mg twice daily

Alfred R. Johnson MD

Johnson D.O., Alfred R.

Order Requested By: Johnson Medical Associates	
Johnson D.O., Alfred R. 997 Hampshire Lane Richardson, TX 75080-8105 Phone: (972) 479-0400	
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License #:	0BF8525
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Phone:	(972) 479-0400
Address:	997 Hampshire LN Richardson, TX 75080-8105
Orders	
Diagnosis:	Z77.120, R53.82, G31.84, F90.9, U09.9, K21.9, G
LOINC:	
Date	Descriptions Instructions
07/07/2025	QEEG QEEG + CNS VS Dx: mold toxicity, brain fog, chronic fatigue

Alfred R. Johnson MD

Johnson D.O., Alfred R.

Order Requested By: Johnson Medical Associates

Johnson D.O., Alfred R.
997 Hampshire Lane
Richardson, TX 75080-8105
Phone: (972) 479-0400

Provider

License #: 0BF8525

Patient

Name: 91408794 - Copeland, Kathryn M.
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Address: 405 Crawford St #2145
Fort Worth, TX 76104

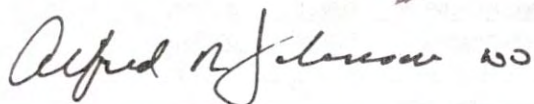
Service Facility

Name: Johnson Medical Assoc., P.A.
Phone: (972) 479-0400
Address: 997 Hampshire LN
Richardson, TX 75080-8105

Orders

Diagnosis: Z77.120, R53.82, G31.84, F90.9, U09.9, K21.9, G
LOINC:

Date	Descriptions Instructions
07/07/2025	Hyperbaric Oxygen Treatment 40 hr HBOT - start @ 1.5 ATM for 60 mins and then increase to 2.0ATM for 60 mins up to twice daily or 90 mins once daily Repeat urine mycotoxins after 10 hrs of HBOT



Johnson D.O., Alfred R.

Kathryn Copeland



PATIENT Patient Lab Bill

DATE: 7.7.25

DX CODE	Price	Test #	Description	DX CODE	Price	Test #	Description
36410	\$3.00		Venipuncture: MEDICARE	E03.8	\$60.00	001974	T4 Free: Serum
36415	\$20.00		Venipuncture	J30.89	\$420.00	676577	Texas Rast
A49.9	\$75.00	8649	Aerobic Bacterial Culture	R53.82	\$207.00	140103	Testosterone Free
M62.81	\$92.00	2030	Aldolase	R53.82	\$82.00	004226	Testosterone: Serum
R19.7	\$60.00	82150	Amylase	J06.9	\$35.00	008342	Throat Culture
M25.50	\$42.00	164855	ANA: Serum	E03.8	\$40.00	004259	TSH: High Sens. Serum
E03.8	\$80.00	006676	Anti-Thyroid Peroxidase	N39.0	\$14.00	003038	UA: Routine
M62.81	\$225.00	120220	C1 Est. Inh. Func., Serum	J06.9	\$35.00	008342	Upper Resp. Culture
K59.00	\$310.00	163135	Candida Antibodies	M10.00	\$18.00	1057	Uric Acid
R53.82	\$12.90	005009	CBC w/ Diff. & Plts	N39.0	\$35.00	008847	Urine Culture
R19.7	\$208.00	165118	Celiac Disease ABS	D51.0	\$35.20		Vit B12 Serum
M62.81	\$240.00	001941	CH50	E55.9	\$160.00	081950	Vit D 25 Hydroxy
R53.82	\$68.00	104018	Cortisol AM, Serum				
R53.82	\$68.00	004051	Cortisol Level: Serum	E80.20	\$160.00	003194	24 Hr. Porphyrins Urine
R53.82	\$68.00	104026	Cortisol PM: Serum				
M62.81	\$45.00	001362	CPK	86003X	\$12.00	86003X	Allergens (IgE,IgG)
R10.84	\$36.00	310900	CMP13				
J01.90	\$35.00	006627	CRP, Quantitative: Serum	DDTUP	\$200.00	DDTUP	Urine: Toxic Metals Profile
R53.82	\$185.00	004697	DHEA Sulfate: Serum	DDRBC	\$310.00	DDRBC	RBC: Trace Minerals
R53.82	\$165.00	216655	EBV Acute Infection: Serum	DDHAIR	\$160.00	DDHAIR	Hair Analysis
J01.90	\$17.00	005215	ESR	DD-CSA	\$375.79	DD-CSA	CDSA
Z79.890	\$120.00	140244	Estradiol,Sensitivity: Serum	DDNBA	\$350.95	DDNBA	Neurotransmitters
Z79.890	\$99.00	004549	Estrogens: Serum				
	\$140.00	008482	Fungal Culture				
R10.84	\$58.00	80058	Hepatic Function Panel				
E11.9	\$75.00	001453	Hgb A1C		\$536.25		GI EFFECTS COMP PROFILE
D51.0	\$110.00	706994	Homocysteine Plasma/Serum	DETOX	\$895.00	DETOX	Genovations Profile
R10.84	\$415.00	163683	H-Pylori IgG/IgM/IgA	GD3001	\$536.25	GD3001	NutrEval
R06.2	\$600.00	660670	Hypersensitivity Pneumonitis				
J30.89	\$60.00	001784	IgA: Serum				
J30.89	\$60.00	002170	IgE Total: Serum				
J01.90	\$60.00	001776	IgG,Quant.: Serum	E8503	\$422.00	E8503	Tricothecenes
J01.90	\$266.00	123225	Immunoglobulin G	E8502X	\$410.00	E8502x	Aflatoxin 1
J01.80	\$60.00	001792	IgM:E77 Serum	E8501	\$410.00	E8501	Ochratoxin
D50.9	\$37.60	001321	Iron & TIBC: Serum	E84000	\$399.00	E84000	All 4 Mycotoxins
D50.9	\$10.40	001339	Iron: Serum	E8400F	\$249.00	83516GP	Mycotoxin Follow up
K85.9	\$59.00	001404	Lipase				
E78.2	\$36.00	303756	Lipid Panel	SP5000	\$448.75	SP5000	FIA 5000 Panel (CASH ONLY)
M25.50	\$35.00	164226	Lyme antibodies				
M25.50	\$200.00	163600	Lyme/Western Blot	81002	\$15.00	In House	Dipstick: Urine
T56.1X1A	\$187.00	085324	Mercury: Blood	87081	\$25.00	In House	Quick Strep
D51.0	\$1,000.00	511238	MTHFR	93000	\$100.00	In House	EKG
Z01.41	\$55.00	009100	Pap Smear, D231 Slide	83036	\$75.00	In House	HbA1C
Z79.890	\$72.00	004317	Progesterone Serum	82948	\$18.00	In House	Glucose Strip
R97.20	\$65.00	010322	PSA: Serum	69210	\$55.00	In House	Ear Lavage
D68.9	\$80.00	020321	PT and PTT	94010	\$80.00	In House	Pulmonary Function Test
M25.50	\$32.00	006502	RA Factor: Serum	96460	\$75.00	In House	Nebulizer Treatment
K59.00	\$104.00	164657	Saccharomyces Panel	96136	\$60.00	In House	Neurocognitive 1 (nurse area)
J01.81	\$536.40	812166	Sero Pneumo 23	96132	\$145.00	In House	Neurocognitive 2 (nurse area)
R10.84	\$55.00	320918	SMAC Panel: Serum				
D68.9	\$366.00	505370	T&B NK Cells Panel				
E03.8	\$93.45	010389	T3 Free: Serum				
DIAGNOSIS CODES:	\$116.00	164068	Covid-19 Nucleocapsid				
	\$116.00	164090	Covid-19 Titer				

Tel: 972-479-0400

997 Hampshire Lane, Richardson, TX 75080

Johnson Medical Assoc., P.A.
 997 Hampshire LN
 Richardson, TX 75080-8105
 (972) 479-0400

IF PAYING BY CREDIT, FILL OUT BELOW			
☐ MasterCard	☐ Visa	☐ Discover	☐ American Express
Card Number		Amount	
Signature		Expiration Date	

Amount(s) Enclosed:

91408794 - Kathryn M. Copeland - Office Visit

Kathryn M. Copeland
 405 Crawford St #2145
 Fort Worth, TX 76104

Statement ID Statement Date Statement Message

21795958 07/07/2025 We appreciate you as a patient here at Johnson Medical Associates.
 Please call to make payment arrangements.

Patients Included on Statement:

91408794 - Kathryn M. Copeland - 405 Crawford St #2145, Fort Worth, TX 76104

Date	Provider	Procedure Code	Description	Charge	Credit	Estimated Insurance	Patient Balance
			Kathryn M. Copeland (91408794) - Office Visit Last Patient Payment: 6/30/25 - 220.00				
7/7/25	1 - Johnson	99203-25	Office/Outpatient Visit, Moderate, New Patient 7/7/25 - Visa 02/26 - 9825 - Payment	220.00	220.00		0.00
7/7/25	1 - Johnson	96136	Psych/Neuropsych test administration	70.00			70.00
7/7/25	1 - Johnson	95816	EEG, awake or drowsy	430.00			430.00
			<u>Amount Due for Kathryn M. Copeland (91408794) - Office Visit</u>				<u>500.00</u>

Combined Patient Responsibility				
Current	Over 30	Over 60	Over 90	Total
500.00	0.00	0.00	0.00	500.00

Total Amount Due	500.00
-------------------------	---------------

Company: Richard Herrscher, MD

Patient: Kathryn Copeland

E-Mail: [REDACTED]

Sex: [REDACTED]

Accession #: 4025010900

Collected: 06/18/2025

Received: 06/26/2025

Completed: 06/27/2025

Procedure Type: Semi-Quantitative ELISA (Enzyme-Linked Immunosorbent Assay)

Code	Test - Urine	Result	Value (ppb)	Reference Range
OCHRA	Ochratoxin A	Present	2.148	<1.8 Not Present 1.8 to <2 Equivocal >=2 Present
AFLA	Aflatoxin Group: (B1, B2, G1, G2)	Present	1.789	<0.8 Not Present 0.8 to <1 Equivocal >=1 Present
TRICHO	Trichothecene Group (Macrocyclic): Roridin A, Roridin E, Roridin H, Roridin L-2, Verrucaric Acid, Verrucaric Acid, Satratoxin G, Satratoxin H, Isosatratoxin F	Present	0.189	<0.07 Not Present 0.07 to <0.09 Equivocal >=0.09 Present
GLIO	Gliotoxin Derivative	Present	1.398	<0.5 Not Present 0.5 to <1 Equivocal >=1 Present
ZEA	Zearalenone	Present	1.158	<0.5 Not Present 0.5 to <0.7 Equivocal >=0.7 Present

About These Tests:

Test results should be evaluated in relation to patient symptoms, clinical history, and other laboratory findings. Individuals should review their results with a healthcare provider.

These tests were developed and the performance characteristics determined by RealTime Laboratories.

They have not been cleared or approved by the U.S. Food and Drug Administration (FDA).

This laboratory is certified under the Clinical Laboratory Improvement Amendments (CLIA) as qualified to perform high complexity clinical laboratory testing.

The New York State (NYS) Department of Health (DOH) has allowed these tests to be offered in the NYS under the current RealTime Laboratories permit.

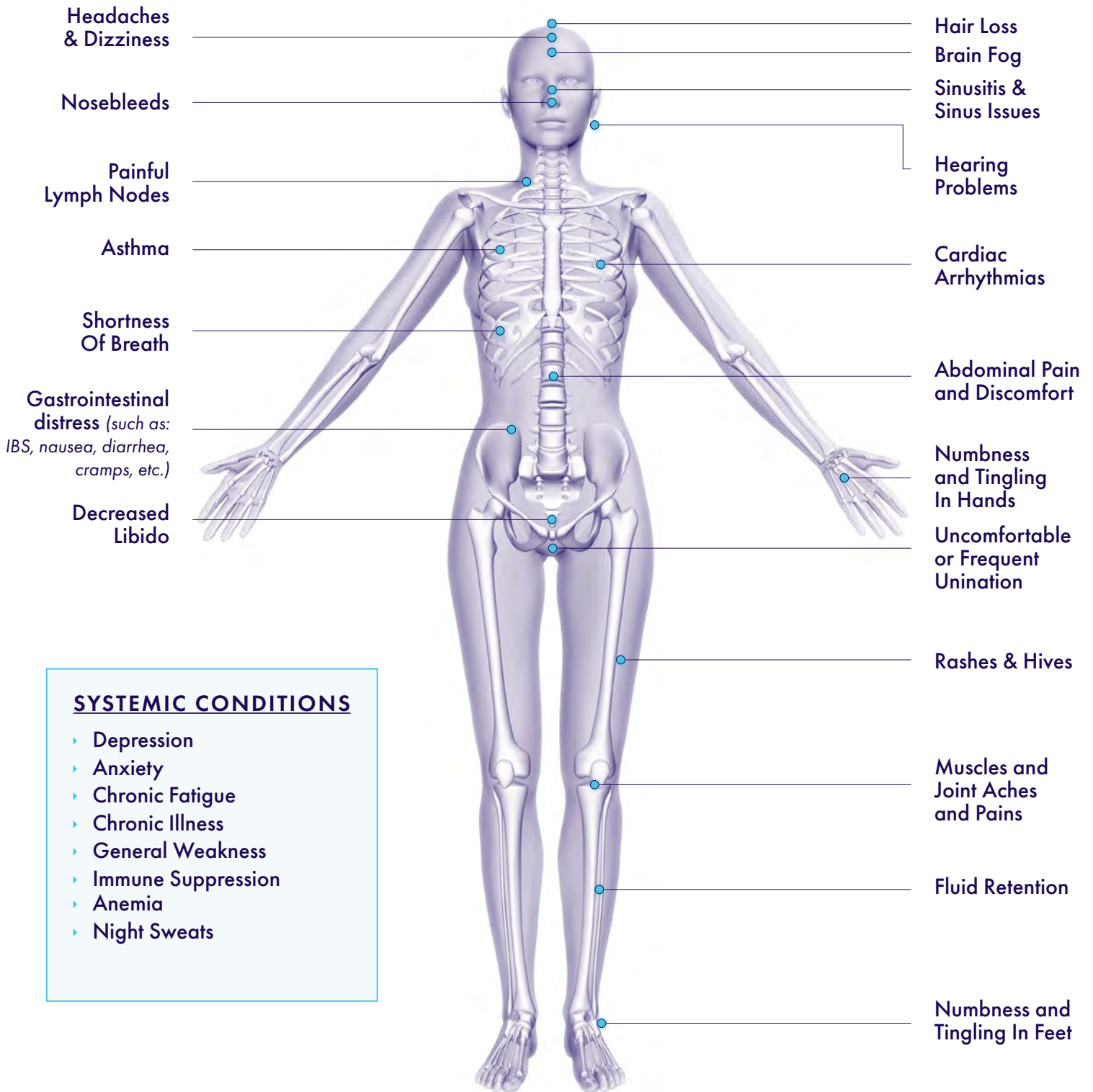
The NYS DOH has not evaluated any test claims nor reviewed the accuracy of these tests.

Mycotoxin		Cellular Activity of Mycotoxin	Symptoms/Other	Association with a "Disease State"
A FLATOXIN FAMILY Organisms: <i>Aspergillus flavus</i>, <i>Aspergillus oryzae</i>, <i>Aspergillus fumigatus</i>, <i>Aspergillus parasiticus</i> Aflatoxins have been associated with liver cancer [2,3], cirrhosis [4,5], and other health issues				
1	Aflatoxin B1	Binds DNA and proteins [6,7]	Shortness of breath [8], weight loss [10], most potent and highly carcinogenic.	Primarily attacks the liver, other organs include kidneys and lungs [11]
2	Aflatoxin B2	Inhibits DNA and RNA replication [12]	Impaired fetal growth [13,14]	Affects the liver and kidneys [11]
3	Aflatoxin G1	Cytotoxic, induces apoptosis in cells, DNA damage [1]	A flavus is a leading cause of invasive aspergillus in immunocompromised patients [15]	Cancer, neonatal jaundice [2,3,16]
4	Aflatoxin G2	Cancer, neonatal jaundice [2,3,16]	Aflatoxicosis in humans and animals [15]	Malnutrition, lung cancer [2,3,16]
OCHRATOXIN A Organisms: <i>Aspergillus ochraceus</i>, <i>Aspergillus niger</i>, <i>Penicillium species</i>				
5	Ochratoxin A	Inhibits mitochondrial ATP, potent teratogen, and immune suppressor [17-19]	Fatigue, dermatitis, irritated bowel [20-22]	Kidney disease and cancer [23,24]
MACROCYCLIC TRICHOPECENES (Group D) Organism: <i>Stachybotrys chartarum</i>				
6	Satratoxin G	DNA, RNA, and protein synthesis inhibition [25]	Fatigue [26]	Bleeding disorders, nervous system disorders [27,28]
7	Satratoxin H	Inhibits protein synthesis [25]	Fatigue [26]	Breathing issues [29]
8	Isosatratoxin F	Immunosuppression [30]	Weakened immune system [30]	
9	Roridin A	Immunosuppression [30]	Weakened immune system [30]	
10	Roridin E	DNA, RNA, and protein synthesis disruption [25,32]	Weakened immune system [30]	Lung and nasal olfactory problems [31]
11	Roridin H	Inhibits protein synthesis [25]	Weakened immune system [30]	
12	Roridin L-2	Immunosuppression [30]	Weakened immune system [30]	
13	Verrucarins A	Immunosuppression [30]		
14	Verrucarins J	Immunosuppression [30]		
GLIOTOXIN DERIVATIVE Organisms: <i>Aspergillus fumigatus</i>, <i>Aspergillus terreus</i>, <i>Aspergillus niger</i>, <i>Aspergillus flavus</i>				
15	Gliotoxin	Attacks intracellular function in immune system [34]	Memory and breathing issues [35,36]	Immune dysfunction disorders [34]
ZEARALENONE Organisms: <i>Fusarium species</i>				
16	Zearalenone	Estrogen mimic [37,38]	Early puberty, low sperm counts, cancer [39-42]	Cancer [39,40]

REFERENCES:

- Zhang, Z., et al., *Cytochrome P450 2A13 is an efficient enzyme in metabolic activation of aflatoxin G1 in human bronchial epithelial cells*. Arch Toxicol, 2013. 87(9): p. 1697-707.
- Wang, S.H., S.H. Yeh, and P.J. Chen, *Androgen Enhances Aflatoxin-induced Genotoxicity and Inflammation to Liver Cancer in Male Hepatitis B Patients*. Cell Mol Gastroenterol Hepatol, 2023. 15(2): p. 507-508.
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SYMPTOMS OF Mycotoxins



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External ID: 191240
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Collected: 06/19/2025
Received: 06/30/2025
Completed: 07/02/2025

canine

Procedure Type: Semi-Quantitative ELISA (Enzyme-Linked Immunosorbent Assay)

Code	Test - Pet Urine	Result	Value (ppb)	Reference Range
OCHRA	Ochratoxin A	Present	2.358	<1.8 Not Present 1.8 to <2 Equivocal >=2 Present
AFLA	Aflatoxin Group: (B1, B2, G1, G2)	Present	1.652	<0.8 Not Present 0.8 to <1 Equivocal >=1 Present
TRICHO	Trichothecene Group (Macrocyclic): Roridin A, Roridin E, Roridin H, Roridin L-2, Verrucarin A, Verrucarin J, Satratoxin G, Satratoxin H, Isosatratoxin F	Present	0.184	<0.07 Not Present 0.07 to <0.09 Equivocal >=0.09 Present
GLIO	Glutotoxin Derivative	Present	1.182	<0.5 Not Present 0.5 to <1 Equivocal >=1 Present
ZEA	Zearalenone	Not Present	0.484	<0.5 Not Present 0.5 to <0.7 Equivocal >=0.7 Present

RTL maintains liability limited to cost of analysis. Interpretation of the data contained in this report is the responsibility of the client.
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 The above test report relates only to the items tested. RTL bears no responsibility for sample collection activities or analytical method limitations.

A FLATOXIN FAMILY			
Organisms: <i>Aspergillus flavus</i> , <i>Aspergillus oryzae</i> , <i>Aspergillus fumigatus</i> , <i>Aspergillus parasiticus</i> Aflatoxins have been associated with liver cancer [2,3], cirrhosis [4,5], and other health issues			
1	Aflatoxin B1	Binds DNA and proteins [6,7]	Shortness of breath [8], weight loss [10], most potent and highly carcinogenic.
2	Aflatoxin B2	Inhibits DNA and RNA replication [12]	Impaired fetal growth [13,14]
3	Aflatoxin G1	Cytotoxic, induces apoptosis in cells, DNA damage [1]	A flavus is a leading cause of invasive aspergillus in immunocompromised patients [15]
4	Aflatoxin G2	Cancer, neonatal jaundice [2,3,16]	Aflatoxicosis in humans and animals [15]
OCHRATOXIN A			
Organisms: <i>Aspergillus ochraceus</i> , <i>Aspergillus niger</i> , <i>Penicillium species</i>			
5	Ochratoxin A	Inhibits mitochondrial ATP, potent teratogen, and immune suppressor [17-19]	Fatigue, dermatitis, irritated bowel [20-22]
MA CROCyclic TRICHOThECENES (Group D)			
Organism: <i>Stachybotrys chartarum</i>			
6	Satratoxin G	DNA, RNA, and protein synthesis inhibition [25]	Fatigue [26]
7	Satratoxin H	Inhibits protein synthesis [25]	Fatigue [26]
8	Isosatratoxin F	Immunosuppression [30]	Weakened immune system [30]
9	Roridin A	Immunosuppression [30]	Weakened immune system [30]
10	Roridin E	DNA, RNA, and protein synthesis disruption [25,32]	Weakened immune system [30]
11	Roridin H	Inhibits protein synthesis [25]	Weakened immune system [30]
12	Roridin L-2	Immunosuppression [30]	Weakened immune system [30]
13	Verrucarin A	Immunosuppression [30]	
14	Verrucarin J	Immunosuppression [30]	
GLIOTOXIN DERIVATIVE			
Organisms: <i>Aspergillus fumigatus</i> , <i>Aspergillus terreus</i> , <i>Aspergillus niger</i> , <i>Aspergillus flavus</i>			
15	Gliotoxin	Attacks intracellular function in immune system [34]	Memory and breathing issues [35,36]
ZEARALENONE			
Organisms: <i>Fusarium species</i>			
16	Zearalenone	Estrogen mimic [37,38]	Early puberty, low sperm counts, cancer [39-42]
Cancer [39,40]			

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- Massart, F. and G. Saggese, *Oestrogenic mycotoxin exposures and precocious pubertal development*. Int J Androl, 2010. 33(2): p. 369-76.

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Completed: 06/27/2025

Canine

Procedure Type: Semi-Quantitative ELISA (Enzyme-Linked Immunosorbent Assay)

Code	Test - Pet Urine	Result	Value (ppb)	Reference Range
OCHRA	Ochratoxin A	Present	2.696	<1.8 Not Present 1.8 to <2 Equivocal >=2 Present
AFLA	Aflatoxin Group: (B1, B2, G1, G2)	Not Present	0.243	<0.8 Not Present 0.8 to <1 Equivocal >=1 Present
TRICHO	Trichothecene Group (Macrocyclic): Roridin A, Roridin E, Roridin H, Roridin L-2, Verrucarin A, Verrucarin J, Satratoxin G, Satratoxin H, Isosatratoxin F	Present	0.140	<0.07 Not Present 0.07 to <0.09 Equivocal >=0.09 Present
GLIO	Glutotoxin Derivative	Present	1.494	<0.5 Not Present 0.5 to <1 Equivocal >=1 Present
ZEA	Zearalenone	Not Present	0.451	<0.5 Not Present 0.5 to <0.7 Equivocal >=0.7 Present

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Mycotoxin		Cellular Activity of Mycotoxin	Symptoms/Other	Association with a "Disease State"
A FLATOXIN FAMILY Organisms: <i>Aspergillus flavus</i>, <i>Aspergillus oryzae</i>, <i>Aspergillus fumigatus</i>, <i>Aspergillus parasiticus</i> Aflatoxins have been associated with liver cancer [2,3], cirrhosis [4,5], and other health issues				
1	Aflatoxin B1	Binds DNA and proteins [6,7]	Shortness of breath [8], weight loss [10], most potent and highly carcinogenic.	Primarily attacks the liver, other organs include kidneys and lungs [11]
2	Aflatoxin B2	Inhibits DNA and RNA replication [12]	Impaired fetal growth [13,14]	Affects the liver and kidneys [11]
3	Aflatoxin G1	Cytotoxic, induces apoptosis in cells, DNA damage [1]	A flavus is a leading cause of invasive aspergillus in immunocompromised patients [15]	Cancer, neonatal jaundice [2,3,16]
4	Aflatoxin G2	Cancer, neonatal jaundice [2,3,16]	Aflatoxicosis in humans and animals [15]	Malnutrition, lung cancer [2,3,16]
OCHRATOXIN A Organisms: <i>Aspergillus ochraceus</i>, <i>Aspergillus niger</i>, <i>Penicillium species</i>				
5	Ochratoxin A	Inhibits mitochondrial ATP, potent teratogen, and immune suppressor [17-19]	Fatigue, dermatitis, irritated bowel [20-22]	Kidney disease and cancer [23,24]
MA CROCyclic TRICHOthECENES (Group D) Organism: <i>Stachybotrys chartarum</i>				
6	Satratoxin G	DNA, RNA, and protein synthesis inhibition [25]	Fatigue [26]	Bleeding disorders, nervous system disorders [27,28]
7	Satratoxin H	Inhibits protein synthesis [25]	Fatigue [26]	Breathing issues [29]
8	Isosatratoxin F	Immunosuppression [30]	Weakened immune system [30]	
9	Roridin A	Immunosuppression [30]	Weakened immune system [30]	
10	Roridin E	DNA, RNA, and protein synthesis disruption [25,32]	Weakened immune system [30]	Lung and nasal olfactory problems [31]
11	Roridin H	Inhibits protein synthesis [25]	Weakened immune system [30]	
12	Roridin L-2	Immunosuppression [30]	Weakened immune system [30]	
13	Verrucarín A	Immunosuppression [30]		
14	Verrucarín J	Immunosuppression [30]		
GLIOTOXIN DERIVATIVE Organisms: <i>Aspergillus fumigatus</i>, <i>Aspergillus terreus</i>, <i>Aspergillus niger</i>, <i>Aspergillus flavus</i>				
15	Gliotoxin	Attacks intracellular function in immune system [34]	Memory and breathing issues [35,36]	Immune dysfunction disorders [34]
ZEARALENONE Organisms: <i>Fusarium species</i>				
16	Zearalenone	Estrogen mimic [37,38]	Early puberty, low sperm counts, cancer [39-42]	Cancer [39,40]

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Patient Name: Kathryn Copeland

Date: September 11, 2025

Provider Name & Credentials: Kim Tran, M.D.

Facility: UT Southwestern Medical Center
600 S Main St 3rd Floor, Suite 500
Fort Worth, TX 76104

To Whom It May Concern,

I am the primary care physician for Kathryn Copeland. She has been under my care for multiple chronic medical conditions that significantly impair her daily functioning, including hypermobile Ehlers-Danlos Syndrome (hEDS), Postural Orthostatic Tachycardia Syndrome (POTS), Mast Cell Activation Syndrome (MCAS), [REDACTED] immune system abnormalities (hypergammaglobulinemia), [REDACTED] and complications from toxic mold exposure.

These conditions collectively cause severe fatigue, post-exertional malaise, autonomic instability, chronic pain, tremors, migraines, visual disturbances, and cognitive dysfunction (slowed processing, memory impairment, and brain fog). Even minimal exertion can trigger symptom flares requiring extended rest.

Since early 2025, Ms. Copeland's health has worsened. She was displaced from her home in June/July 2025 due to toxic mold and has been forced to live in unstable hotel housing. This disruption prevents her from accessing prescribed home health services, therapy, and ongoing medical care, further compromising her health.

[REDACTED]

[REDACTED]

[REDACTED]

Sincerely,

Kim Tran, M.D.
UT Southwestern Medical Center

University Hospital Physical Medicine & Rehabilitation Clinic
Clinic Phone: 214-645-2080

September 8, 2025

Patient Name: Kathryn Copeland
[REDACTED]

RE: Medical Statement for Disability Application

To Whom It May Concern,

[REDACTED]
[REDACTED] She has been under my care at UTSouthwestern since October 8, 2024, for Hypermobility EhlersDanlos Syndrome (hEDS), Mast Cell Activation Syndrome (MCAS) and Postural Orthostatic Tachycardia Syndrome (POTS). These are severe chronic illnesses that significantly impair her daily functioning [REDACTED]

In addition to these primary conditions, review of her medical records shows she is also being treated for the following chronic and compounding illnesses: [REDACTED]

[REDACTED] Hashimoto's disease with severely low iron requiring three iron infusions, bilateral vision loss and binocular dysfunction linked to toxic mold exposure, [REDACTED] and immune abnormalities including hypogammaglobulinemia.

Her July 2025 evaluation by Dr. Alfred Johnson documented toxic mold exposure with extremely elevated levels of Aspergillus, Chaetomium, and Stachybotrys. He noted severe chronic fatigue (up to 18 hours/day bedridden), 30 lbs of weight loss, cognitive impairment, and hyperbaric oxygen therapy and urgent need for relocation.

Since my prior March 2025 statement, Ms. Copeland's health has further deteriorated. She was displaced in June/July 2025 due to mold contamination and has been forced to live in a hotel. This unstable housing situation prevents her from accessing home health nursing and supportive services previously prescribed, further worsening her decline.

She continues to experience severe and disabling symptoms, including: • Severe systemic inflammation.

- Persistent fatigue and post-exertional malaise.
- Orthostatic intolerance, dizziness, tremors, and frequent migraines.
- Neurological and sensory issues, including visual disturbances and light sensitivity.
- Severe GI impairment with ongoing nutritional instability.
- Cognitive dysfunction, including impaired memory, slowed processing, and executive dysfunction.

This

lack of resources is directly worsening her health and preventing her from accessing prescribed care.

In my professional opinion, Ms. Copeland remains unable to perform sustained work activity and is in urgent need of financial and medical support to stabilize her living conditions and prevent irreversible decline.

Sincerely,

 9/10/25

Isabel Huang, MD
UT Southwestern Medical Center

**Michael Conte, O.D. -
Neurology Opthamologist**

June 13, 2025

EXAMINATION RECORD

Lake Worth Vision Source
6636 Lake Worth Blvd Suite 300
Lake Worth, TX 76135-3002
817-626-4441 FAX: 817-625-7675

For: Copeland, Kathryn M

Exam Date: 06/13/2025

Print Date: 06/13/2025 10:44 AM

Age: 39

Occupation: Student TCU-britedivinity

Gender: Female **Race:** White

CHIEF COMPLAINT

NOTES: WAS PATIENT REFERRED:yes,christi furguson

TYPE OF INJURY AND DATE: TBI aquired brain injury- 2 years ago

HAS PATIENT BEEN IN PT/OT IF SO HOW LONG:yes both- a couple weeks ago

ANY VISUAL SYMPTOMS LOSS OF PERIPHERAL VISION, DISTORTION, DOUBLE VISION:yes peripheral loss, distortion - cant see the tv sometimes strains eyes, vision changes a lot.

LOSS OF MOBILITY:yes

HISTORY PRESENT ILLNESS (HPI)

VISION COMPLAINT: Bilateral: With current spectacle correction Problems are experienced primarily at distance.

INTEGUMENTARY: No symptoms reported at this time.
MUSCULOSKELETAL: No symptoms reported at this time.
NEUROLOGICAL: No symptoms reported at this time.
PSYCHIATRIC: No symptoms reported at this time.
RESPIRATORY: No symptoms reported at this time.

PRESENTING FINDINGS

PRESENTING SPECTACLE Rx: (#1)

RT: DVA: 20/20-
LT: DVA: 20/25

VISION

K-READINGS:

RT: 38.00 @ 098 Steep 38.50 @ 008
LT: 38.25 @ 111 Steep 38.75 @ 021

NOTES: Eye Movement Skills:

Extraocular motility:
Pursuits:
Saccades:
Cover test distance:4x
Cover test near:8x
NPC: Date-time: 6/13/2025 9:55:36 AM By: Mehallic, Mackenzie

AUTO REFRACTION:

RT: Plano -1.00 x 071
LT: Plano -0.75 x 094

FINAL SPECTACLE Rx:

RT: -0.25 -1.00 x 080 HPrism: 1.00BI Add: +1.00 DVA: 20/20
LT: -0.25 -0.75 x 105 HPrism: 1.00BI Add: +1.00 DVA: 20/20

EXAMINATION

CUP/DISC RATIO:

RT: Horz .40 Vert .40
LT: Horz .40 Vert .40

TONOMETRY: RT: 9 mmHg LT: 9 mmHg Test: Non-Contact Time: 09:17 AM Category: Examination

SLIT-LAMP EXAM: punctate staining.

SPECIAL TESTING

EXTERNAL PHOTOGRAPHY: Bilateral: CPT 92285 - Interpretation and Report: Anterior segment imaging demonstrates a defective ocular structure in the following areas:

OPTOS Retinal Imaging: Bilateral: CPT 92250 - Interpretation and Report:

SCANNING LASER OPHTHALMOSCOPY (NFL): Bilateral: CPT 92133 - Interpretation and Report:

FIELD STUDIES-General (92083): Bilateral: CPT 92083 - Interpretation and Report:

SENSORIMOTOR EXAM: Bilateral: Sensory defect in visual acuity found.

IMPRESSION(S):

Right Eye: tumor

Bilateral: toxic mold mycotoxin exposure

Bilateral: Dry eye syndrome

Bilateral: O/C due to CI and Binocular Issues due to exposure to toxins

PLAN

TREATMENT NEUROLOGY: Bilateral: Future VEP

TREATMENT DRY EYE: Bilateral: schedule 3x heat tx

SPECTACLE PLAN: Bilateral: Rx spectacles. Prism.

PATIENT MANAGEMENT

COUNSELING: Counseling has been provided to review this patient's case and discuss options for treatment.

COUNSELING / EDUCATION: I have verbally discussed my clinical findings and recommendations in detail with this patient and/or parents. They acknowledge that they do not have additional questions.

ELECTRONIC SIGNATURE: Electronically Signed By: Michael D. Conte, O.D. on 06/13/2025 10:01 AM.

DIAGNOSIS:

H04.123 Dry eye syndrome of bilateral lacrimal glands

S06.0X0A Concussion without loss of consciousness, initial encounter

PROCEDURE:

99214 Level 4 .Estab

92250 Photography-fundus

92285 Photography-external

92060 Therapy-vis-sensorimotor

92133 GDX Scanning Laser

92083 Fields Visual-30-2/goldmn



Completed Exam:

Michael Conte, O.D.

Date: 06/13/2025

DIOPSYS® NOVA-LX
OFFICE BASED NEURO OPTIC VISION ASSESSMENT

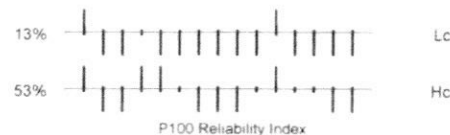
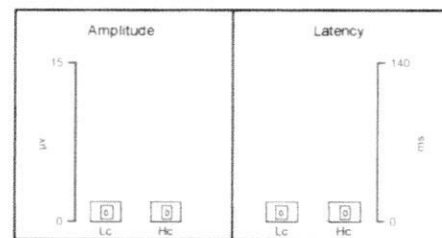
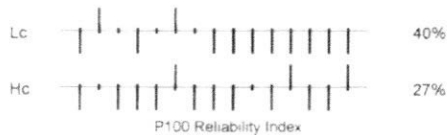
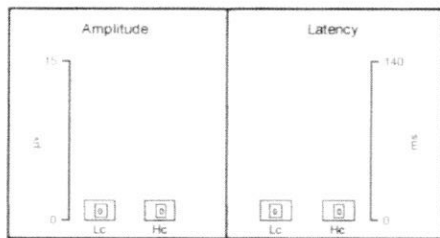
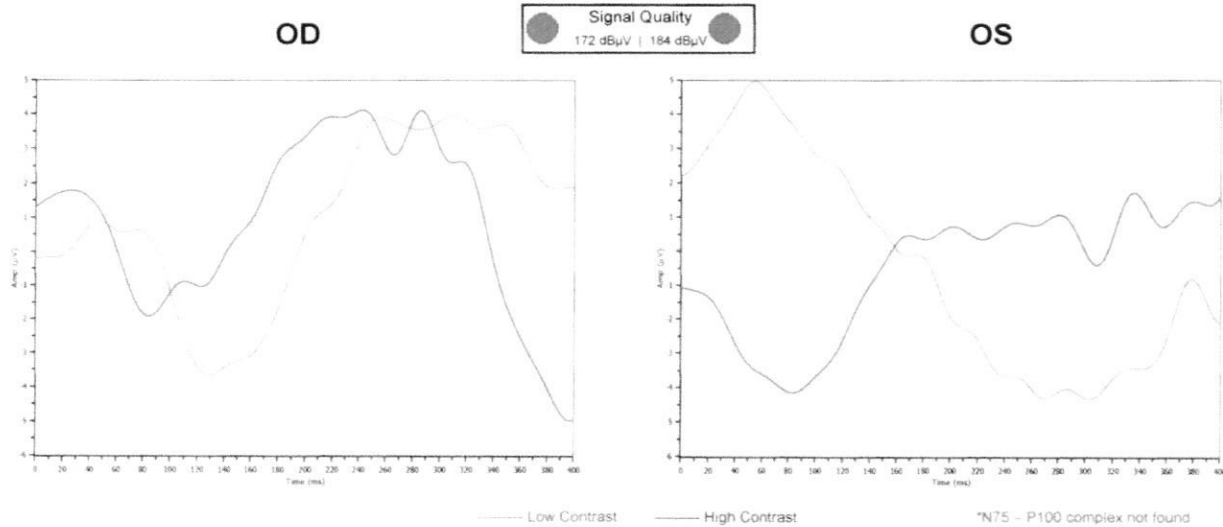
First Name: KATHRYN
Last Name: COPELAND
Patient ID:

DOB: [REDACTED]
Age: 38
Gender: Female

Exam Date: 2024-06-14
Exam Time: 05:45:35 PM

OD: S/C/Ax/Ad:/0.00/0/0.00
OS: S/C/Ax/Ad:/0.00/0/0.00

VA: 20/25
VA: 20/25



Parameters	OD	OS	Difference
Amplitude Low Contrast μ V	*	*	--
Amplitude High Contrast μ V	*	*	--
Latency Low Contrast ms	*	*	--
Latency High Contrast ms	*	*	--

* - P100 not identified, abnormal VEP response

Remarks
* P100 not identified
* P100 not identified
* P100 not identified
* P100 not identified

Operator: Solis, Sandra

Michael Conte, OD - 3855

Comments:

Signature:

OD |

kathryn copeland	06/14/2024	N/A	24
952963 [REDACTED] female	3:48 PM	III	AVASStandard

4.29%

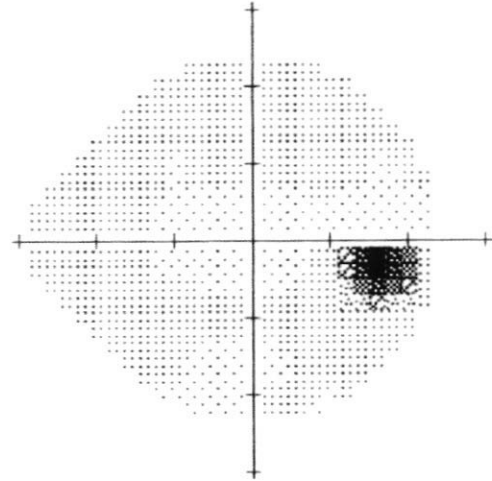
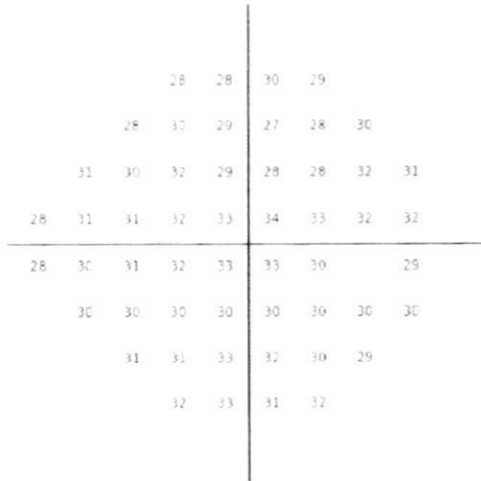
N/A

0/4

2:59

N/A

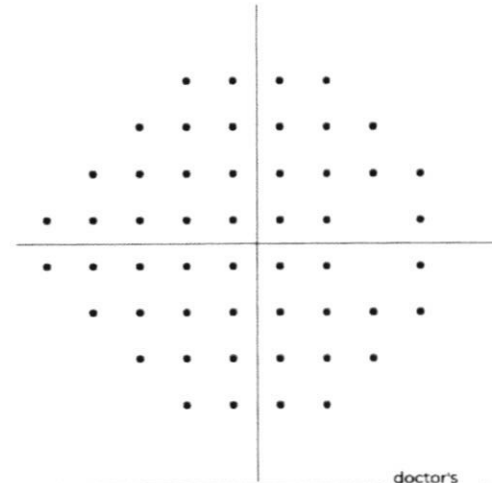
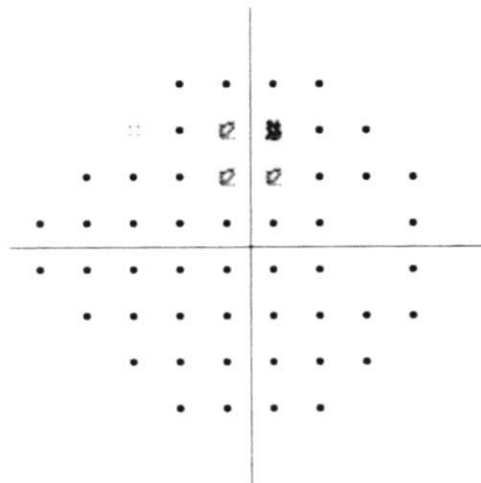
0/4



Total Deviation

MD: -0.84
PSD: 2.10
VFI: 100

Pattern Deviation



Lake Worth Vision Source 6636 Lake Worth Blvd., Suite #300 TX 76135	Mehallic, Mackenzie	doctor's signature Conte, Michael
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VisuALL 2024: AVASStandard is a threshold test. Final diagnosis is doctor's responsibility.

| OS

kathryn copeland	06/14/2024	N/A	24
952963 XXXXXXXXXX female	3:48 PM	III	AVASStandard

11.19%

N/A

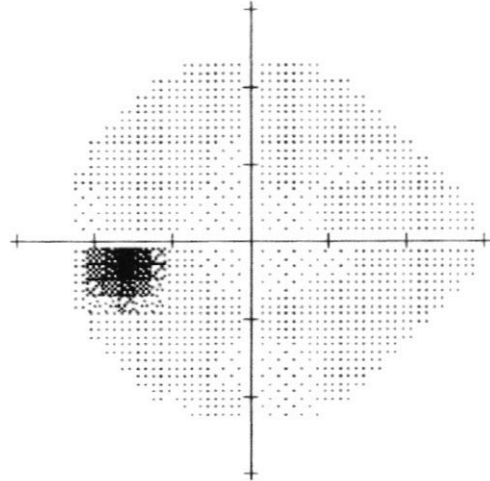
0/4

2:55

N/A

3/4

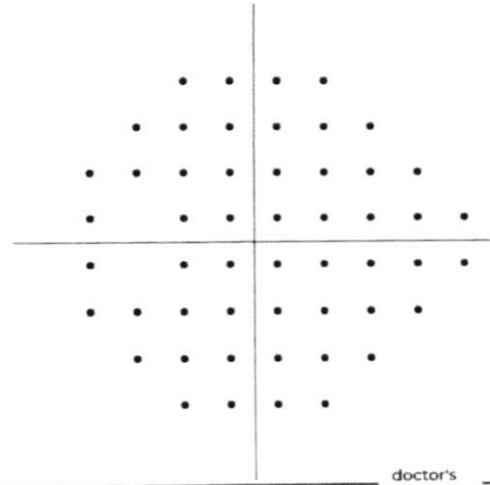
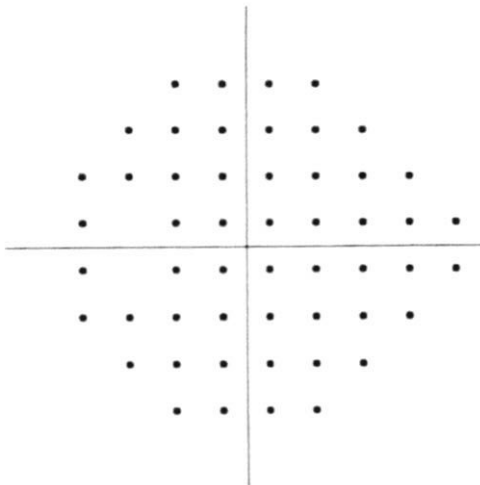
			29	30	28	28			
		30	30	31	31	30	29		
31	31	29	32		30	32	32	31	
32	31	31	33		33	31	31	29	28
29		30	33		33	32	31	30	28
30	30	30	30		30	30	30	30	
	29	30	32		33	31	29		
		32	31		33	31			



Total Deviation

MD: -0.70
PSD: 1.80
VFI: 100

Pattern Deviation



Lake Worth Vision Source 6636 Lake Worth Blvd., Suite #300 TX. 76135	Mehallic, Mackenzie	Conte, Michael	doctor's signature
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VisuALL 2024: AVASStandard is a threshold test. Final diagnosis is doctor's responsibility.

Vision Source

Stacey Sykes

Optician

Town & Country Vision Source
2530 Jacksboro Hwy.
Fort Worth, TX 76114
817-626-2020
Fax: 817-720-6338

Lake Worth Vision Source
6636 Lake Worth Blvd., Suite 300
Lake Worth, TX 76135-3002
817-626-4441
Fax: 817-625-7675

visit our website at: www.visionsource-drmconte.com

e Worth Vision Source
Michael Conte O.D.
ke Worth Blvd. Suite #300
ike Worth, TX 76135
(817) 626-4441

Quote for Kathryn Copeland (DOB [REDACTED])

Frames--\$520.00 (recommended for her due to weight of frame)
Progressive no - Line \$560.00
Polycarbonate--\$85
Prism--\$40.00
Anti-Reflective--\$220.00
Transitions--\$180.00
Drill Fee--\$40.00
Poshish--\$25.00
Warranty Frame--\$20.00

1,690

Cost of glasses

845 1/2

Exhibit D

**Pages 84-92 (Ex. E) of
Defendant's Emergency Motion to Vacate
Temporary Injunction and Enter Affirmative
Relief by Submission (ADA/FHA – Fraud on
Court – Constructive Eviction)
filed October 23, 2025**

Consists of correspondence to opposing
counsel that detail landlord's duties under
FHA and ADA along with case facts

Subject: Re: Urgent Request for Contact to Arrange Immediate Lodging – FHA Accommodation
Date: Tuesday, September 16, 2025 at 7:51:02 PM Central Daylight Time
From: Copeland, Katie
To: Michelle Sortor
CC: John Douglas, Glynis Zavarelli, 'Brochstein, Amye', Lile Benaicha, Pamela Quinn
Priority: High
Attachments: Hotel Drover .pdf, 9 Tax Exempt Form - color.pdf

Dear Ms. Sortor,

Your client's refusal to fund transport for my belongings to my apartment (per Ms. Zavarelli's call, Sept 11, 2025) led to my arrest for criminal trespass at Bowie House, despite my negotiated deal, and forced me back into a toxic mold unit (BioTex report, June 18, 2025) post-hospitalization, violating FHA § 3604(f)(3)(B). Your Holiday Inn offer fails to accommodate my disabilities (brain injury, MCAS, long COVID, and exacerbated injuries). I am exhausted, hungry, and facing compounding damages from ongoing exposure. *Housons, Inc. v. Township of Brick* (3d Cir. 1997) mandates prompt accommodations, as shown by my medical records and Aug 1, 2025, "Notice of Nonpayment."

I demand Hotel Drover (<https://www.hoteldrover.com/>) (negotiated a 2 week rate at \$549/night, attached) for therapy access, plus meals (\$50–80/day) and transport (\$30/day). The Drover attached a tax-exempt form for a 30+ day stay to save money. Provide a contact by 9:00 PM CDT today, per Texas Property Code § 92.056, or I will escalate my HRC complaint (FW-23-03, filed Aug 14) to HUD and seek a TRO, per *HUD v. Ross* (S.D. Ohio 2016).

Sincerely,

Kathryn Copeland

817-789-8498

From: Michelle Sortor <msortor@wandzlaw.com>
Date: Tuesday, September 16, 2025 at 6:16 PM
To: Copeland, Katie <K.M.COPELAND@tcu.edu>
Cc: John Douglas <john@jadouglaslaw.com>, Glynis Zavarelli <[gzavarelli@wandzlaw.com](mailto:gzarelli@wandzlaw.com)>, 'Brochstein, Amye' <amyebrochstein@phly.com>, Lile Benaicha <lbenacha@weinsteinproperties.com>, Pamela Quinn <pquinn@weinsteinproperties.com>
Subject: RE: Urgent Request for Contact to Arrange Immediate Lodging – FHA Accommodation

[EXTERNAL EMAIL WARNING] DO NOT CLICK LINKS or open attachments unless you recognize the sender and know the content is safe.

Ms. Copeland – I have not had a response. I (and my client) can only be available for a short time period this evening, due to other obligations.

Please let me know immediately which hotel you would prefer of the options provided, so that we can arrange same. If I do not receive a response from you by 6:30 p.m., we will assume that you have secured alternate lodging for the evening.

Regards,

Michelle Sortor

Michelle S. Sortor
Wentz & Zavarelli, L.L.P.
3120 Sabre Drive, Suite 170
Southlake, Texas 76092

(469) 665 – 9100 – Telephone
(469) 262 – 2078 – Direct Line
(469) 665 – 9106 – Facsimile

From: Michelle Sortor

Sent: Tuesday, September 16, 2025 5:49 PM

To: 'Copeland, Katie' <K.M.COPELAND@tcu.edu>

Cc: John Douglas <john@jadouglaslaw.com>; Glynis Zavarelli <gzavarelli@wandzlaw.com>; 'Brochstein, Amye' <amy.e.brochstein@phly.com>; Lile Benaicha <lbenaicha@weinsteinproperties.com>; Pamela Quinn <pquinn@weinsteinproperties.com>

Subject: RE: Urgent Request for Contact to Arrange Immediate Lodging – FHA Accommodation

Ms. Copeland – Weinstein will secure a reservation on your behalf at either the Holiday Inn – Fossil Creek or the Holiday Inn Fort Worth Downtown. Please indicate which of those hotels you would prefer, and I will coordinate to provide you reservation details.

Regards,

Michelle Sortor

Michelle S. Sortor
Wentz & Zavarelli, L.L.P.
3120 Sabre Drive, Suite 170
Southlake, Texas 76092

(469) 665 – 9100 – Telephone
(469) 262 – 2078 – Direct Line
(469) 665 – 9106 – Facsimile

From: Copeland, Katie <K.M.COPELAND@tcu.edu>

Sent: Tuesday, September 16, 2025 4:59 PM

To: Glynis Zavarelli <gzavarelli@wandzlaw.com>; Michelle Sortor <msortor@wandzlaw.com>;
'Brochstein, Amye' <amyebrochstein@phly.com>; Lile Benaicha <lbenaicha@weinsteinproperties.com>;
Pamela Quinn <pquinn@weinsteinproperties.com>

Cc: John Douglas <john@jadouglaslaw.com>

Subject: Urgent Request for Contact to Arrange Immediate Lodging – FHA Accommodation

Importance: High

Dear Weinstein Properties,

I am entitled to reasonable accommodation under 42 U.S.C. § 3604(f)(3)(B) due to my disability, exacerbated by uninhabitable conditions (Texas Property Code § 92.052) confirmed by mold exposure, necessitating immediate alternative lodging. As established in *Hovsons, Inc. v. Township of Brick* (1997), such accommodations must be provided promptly to prevent irreparable harm. *Giebler v. M & B Associates* (3d Cir. 1997) and *Schwarz v. City of Treasure Island* (11th Cir. 2007) further confirm that delays or failures to accommodate health-related housing issues violate FHA, especially with imminent risks. **Please provide the contact person to arrange a safe hotel room tonight**, including funds for transport and meals, and advise on required documentation.

Thank you for your immediate assistance.

Katie Copeland
817-789-8498 - Feel free to call or text

Subject: Re: Urgent ADA/FHA Accommodation – Omni Fort Worth
Date: Thursday, September 18, 2025 at 4:53:43 PM Central Daylight Time
From: Copeland, Katie
To: Glynis Zavarelli, Lile Benaicha, 'Brochstein, Amye', Michelle Sortor, Pamela Quinn
CC: John Douglas
Attachments: image.png, image.png

Each time Weinstein insists on placing me at a hotel that I have already demonstrated is medically ineffective, that is not an “offer” — it is a separate denial of my requested reasonable accommodation under the FHA and ADA. You have been repeatedly notified (in writing and with exhibits) that I am medically fragile, that I lack funds and transportation to move my medical equipment, and that generic hotel placements are harmful. Those repeated refusals are willful, cause ongoing medical harm, and each constitutes its own violation.

TDLR has outlined the factors its prosecutors weigh in sanctioning violations: seriousness, willfulness, good-faith mitigation, history of past violations, deterrence, and justice considerations. Each factor weighs heavily against Weinstein here: you knowingly forced a disabled tenant into a condemned unit, acted in bad faith by repeating ineffective placements, and ignored clear medical documentation. This is not an isolated mistake — it is a pattern.

Finally, as these violations become more egregious, I ask you to clarify: who is actually making these decisions? Is it Weinstein Properties — a housing provider obligated to comply with the FHA? Or is it Ms. Zavarelli as counsel — who has independent duties under federal law and the Texas Rules of Professional Conduct? Either way, accountability will attach directly to the decision-maker.

Accountability and scope of duties

- Housing provider liability: Weinstein, as the housing provider, has the FHA duty to provide an effective, individualized accommodation (42 U.S.C. § 3604(f)(3)(B)) and to avoid interference (§ 3617). Counsel's role does not shield unlawful decisions.
- Individual participation liability: Individuals who personally participate in discriminatory decisions can be liable under the FHA, including for aiding/abetting and interference (see DOJ guidance; cf. *Housons v. Township of Brick*).
- Counsel's professional obligations: I understand counsel's role, but directing or implementing decisions that deny effective accommodation or condition it on unrelated terms risks both FHA exposure and professional-conduct concerns (e.g., Texas Rules 4.04, 8.04).

I have repeatedly compromised and presented concrete proposals for safe, medically effective accommodations. Your responses have been limited to placements already proven ineffective. That is not compliance; it is ongoing discrimination. Compliance requires immediate effective accommodation, not further deflection. Another rejection of my carefully created requests is a knowing and intentional act to subject me to more time in a poisonous environment. At the very least, that would be considered duress in our “interactive process.” So I suggest you approve my request - even if just for a few days - so that we can have a better discussion.

Warmly,
Katie Copeland
817-789-8498

From: Copeland, Katie <K.M.COPELAND@tcu.edu>

Date: Thursday, September 18, 2025 at 1:16 PM

To: Glynis Zavarelli <gzavarelli@wandzlaw.com>, Lile Benaicha <lbenaiacha@weinsteinproperties.com>, 'Brochstein, Amye' <amyebrochstein@phly.com>, Michelle Sortor <msortor@wandzlaw.com>, Pamela Quinn <pquinn@weinsteinproperties.com>

Cc: John Douglas <john@jadouglaslaw.com>

Subject: Urgent ADA/FHA Accommodation – Omni Fort Worth

Dear Weinstein Properties and Counsel,

This is an urgent ADA/FHA accommodation notice. I recognize it may be difficult for others to fully understand the daily impact of my disabilities. I invite you to resolve this collaboratively because **what I am requesting is not optional—it is a statutory requirement.** I have attached a chart to help you understand my specific needs. In case you did not know, **my access to prescribed home therapies and meal delivery services was cut off due to displacement, thus a per diem allowance and medically appropriate lodging are necessary accommodations.** These supports are not luxuries; they are disability-related medical needs. A stable per diem ensures continuity of nutrition, hygiene, and therapeutic care until I am placed in ADA-compliant housing where services can resume.

I am willing to discuss reasonable compromises, but under the Fair Housing Act and Fort Worth Code §17-86, I am entitled to:

- **Safe, ADA-compliant lodging of my choosing, not subject to arbitrary night-to-night extensions.**
- **A per diem or equivalent allowance to replace the loss of in-home therapies, nursing, and meal delivery caused by displacement.**
- Assurance that retaliation will cease and that my housing stability will not be undermined for asserting these rights.

Enforcement & Regulatory Posture (for planning):

- **TDLR:** Investigator Ida Parmer (Investigator IV, Enforcement Division) confirmed this case was forwarded to the prosecution team on August 30; I am coordinating with their office.
- **City of Fort Worth:** I remain in active contact with multiple departments, including Code Compliance, Mayor's Committee on Persons with Disabilities, and the Human Relations Commission. While Code Enforcement has paused action pending TDLR's prosecution, they retain authority to declare the building uninhabitable to protect tenants.
- **Licensure prerequisites:** For ≥ 25 contiguous sq ft, TDLR notification is due ≥ 5 calendar days before remediation; protocol/work plan must be provided ≥ 1 day before any start. **Please provide the notification number, start/stop dates, and current plan in advance.** Ms. Zavarelli recently stated work would begin Monday, 9/22/25, but Investigator Parmer confirmed she has not received anything from The Bowery—meaning work cannot lawfully begin.

Accommodation Needed Today:

- **Hotel:** Omni Fort Worth (or functionally equivalent)
- **Check-in:** Today
- **Duration:** 3 weeks guaranteed, with option to extend through Oct 31, 2025
- **Rooming:** King suite now; transition to two adjoining rooms if my daughters return to my care
- **Supports:** Meal credits for medically appropriate diet; laundry access; portable HEPA

Medical Treatment Access:

Because of your recent refusal to provide effective accommodations, I was re-exposed to toxic mold for a week, worsening my condition. Immediate access to my treating physician, Dr. Johnson (Richardson), is also required. Approving transportation and treatment costs now prevents greater medical and liability exposure later. Four treating physicians have documented that mold exposure and displacement have significantly worsened my health.

Per Diem / Reimbursements:

- **Option A (actuals + prospective):** Reimburse past out-of-pocket food/transport (receipts / statements available) within 5 days. Going forward: \$80/day meals (federal M&IE) + \$30/day local transportation.
- **Option B (continuous per diem):** \$80/day meals and \$30/day local transportation from 6/27/25 through present and continuing until ADA-compliant permanent housing is secured.

Note: Laundry is separate from M&IE and will be reimbursed accordingly.

Deadline:

Please confirm the Omni booking and folio coverage, medical transport to Dr. Johnson, and your selection of Option A or B by **3:00 p.m. CT today, September 17, 2025**. *Identify the decision-maker with full authority*. Absent timely confirmation, I will treat further delay as ongoing discrimination and proceed with HUD intake via the City of Fort Worth and seek emergency relief.

Needs vs. Options

Needs vs. Options matrix (✓ met · Δ partial · ✗ not met)

Legend: ✓ Met · Δ Partial · ✗ Not met

Medical/Functional Need	Holiday Inn Express	Nobleman	Bowie House	Hotel Drover	Crescent	Omni	Permanent Relocation
Bathtub / hot tub (pain, POTS)	✗	✗	Δ (hot tub across hall)	✓	✓	✓	✓
Therapy space (PT/OT/ST + caregivers)	✗	✗	✗	Δ (suite possible or 2 rooms)	✓ Suite or 2 rooms available	✓ Suite or 2 rooms available	✓
Kitchen for anti-inflammatory diet	✗	Δ (limited)	✗	Δ (unknown)	Δ (unknown)	Δ No, but includes breakfast every day (more options than Holiday Inn)	✓
Washer/dryer for hygiene	✗	✗	✗	Δ (No but hotel can do laundry for fee)	Δ (No but hotel can do laundry for fee)	Δ (No but hotel can do laundry for fee)	✓
Clean air / filtration (MCAS)	✗	✗	Δ	✓ (filters/PURE)	Δ (unknown)	✓ More rigorous air maintenance + Portable air purification units	✓
ESA yard/patio (ground access)	✗	✗	✗	✗	✗	✗	✓
Stability (30+ days, not week-to-week)	✗	✗	Δ	✓ (negotiated rate + tax-exempt)	Δ (unknown)	✓ At least 3 weeks - could likely negotiate	✓
Transportation provided / accessible location	✗	✗	✗	Δ (central access)	Δ (nearby restaurants and 7-11)	Δ (nearby restaurants, free breakfast)	✓

Item	Exact ask	Why (medical/legal)	Approve
Hotel	Omni Fort Worth, 21 nights starting tonight; King suite → two adjoining rooms if needed; folio direct-billed	Hydrotherapy, clean air, stability; FHA/ADA effectiveness	☐ Approved
Meals	\$80/day per diem or equivalent meal credits adequate for anti-inflammatory diet	MCAS/Long COVID nutrition; FHA effectiveness	☐ Approved
Local transport	\$30/day stipend while displaced	Access to care/food; not included in M&IE	☐ Approved
Medical transport	Fund round-trip travel to Dr. Johnson (Richardson) for scheduled and follow-ups	Treating physician access post re-exposure	☐ Approved
Laundry	Hotel laundry covered or \$15/day stipend	Hygiene/MCAS triggers during multi-week stay	☐ Approved
HBOT	Authorize hyperbaric oxygen treatments, supplements, testing, etc. per Dr. Johnson's plan	Mitigate injury from re-exposure; prevent irreversible decline, significant cost savings	☐ Approved
Past expenses	Option A: reimburse actuals for food/transport since displacement; or Option B: per diem retro at \$80 + \$30	Stabilize nutrition/transport continuity; fair, simple administration	☐ A ☐ B

I believe these are your only options:

Path	Up-front cost (21 nights)	Medical impact	Legal risk	Total exposure
Approve Omni + supports	Hotel + M&IE + transport + laundry + HBOT	Stabilizes; prevents irreversible decline	Low (meets FHA/ADA duties)	Predictable and limited
Place in HIE-type hotel	Lower room rate; no supports	Harm escalates; ER/relapse risk	High (ineffective accommodation + \$3617)	High and compounding
Delay/deny	Near zero now	Worsening injury; specialty care required	Very high (HUD/DOJ, injunction, damages)	Maximized

Sincerely,

Kathryn (Katie) Copeland

Phone: 817-789-8498 | Email: k.m.copeland@tcu.edu

Legal Citations:

42 U.S.C. § 3604(f)(2) – Discrimination in terms, conditions, or privileges of rental housing

42 U.S.C. § 3604(f)(3)(B) – Failure to make reasonable accommodations when necessary

42 U.S.C. § 3617 – Interference, coercion, or intimidation in exercising rights

Fort Worth Municipal Code §17-86 (Ordinance 11075)

Why I Am Sending This Detail

As a courtesy, I have spelled out not only what accommodations I require but also **why each request is medically and legally necessary**. I am not obligated to provide this level of explanation under the ADA or FHA, but I have done so to streamline the process and make compliance easier for you. The attached chart further summarizes the needs → reasons → legal obligations, so there can be no confusion about what is required. I encourage you to engage counsel familiar with FHA and ADA laws because this ongoing conflict is causing much more harm to my health.

Subject: Urgent: Mold Issue & Health Concerns

Date: Wednesday, March 5, 2025 at 12:52:24 PM Central Standard Time

From: Katie Copeland

To: Bowery Southside

Hello, I never realized mold was so serious. I just asked ChatGPT about the mold and my unique constellation of diseases. It told me this was urgent and suggested I write the following:

I am reaching out to report a concerning mold issue in my apartment. I have noticed mold growing in the corner of my entryway, which raises concerns that there may be mold in other areas as well. Given my medical conditions, this is an urgent issue that could severely impact my health.

I have several chronic illnesses, including mast cell activation syndrome (MCAS), celiac disease, and long COVID, among others, which make me extremely sensitive to environmental triggers like mold. Mold exposure can cause severe allergic reactions, respiratory issues, immune system dysfunction, and increased inflammation, all of which could seriously worsen my condition.

Because of these health risks, I need an inspection as soon as possible to determine the extent of the mold issue and whether remediation is necessary. If testing confirms a mold problem, I will need immediate intervention to ensure my apartment remains a safe living environment.

Please let me know how soon we can schedule an inspection and discuss next steps. I appreciate your prompt attention to this matter, as it directly affects my ability to safely remain in my home.

Separate question: do you know anyone who does dog walking in our complex or nearby? I had a medical procedure done today and it would be great if I could pay someone to walk my dogs since I am not able to move much.

Best,

Katie Copeland
817-789-8498

Exhibit E

Subject: Weinstein v. Copeland

Date: Thursday, October 16, 2025 at 6:27:40 PM Central Daylight Time

From: Glynis Zavarelli

To: Copeland, Katie

CC: Michelle Sortor

[EXTERNAL EMAIL WARNING] DO NOT CLICK LINKS or open attachments unless you recognize the sender and know the content is safe.

Ms. Copeland:

It is my understanding that your current hotel reservation at The Worthington expires on October 18th (meaning that is the day of check out). However, Weinstein is in the process of trying to extend this reservation (for the same room) through October 26- meaning, if it can be extended, you will need to check out on October 27. Weinstein will be paying the costs for the room through the October 26th date (and this does not include payment of incidentals). I will update you regarding the status of this extension, as soon as I receive that information.

Additionally, Weinstein will continue to incur the costs associated with boarding your dogs through October 26; as such, you will need to pick them up from Doggie Diggs on October 27, or if you choose to keep them there from October 27 forward, you will need to pay for their care/boarding.

Further, as previously indicated, all items in both storage rooms (the one associated with your apartment, and the one which is on the 4th floor that is being used to store the items that were moved out of your apartment on October 5) will need to be moved out (at your expense) before October 27, and your keys/access fobs (apartment key fob, mailbox key, storage unit keys, gate access fob) can be placed in the rent drop box (located near the leasing office). To the extent, the items are not moved out on October 26, and/or you do not timely return your keys/access fobs, Weinstein will pursue all necessary legal remedies, and in that regard, I encourage you to review your lease agreement.

Please note that Weinstein's payment of the continued hotel stay and pet boarding through October 26 – is not an admission of liability nor is it an admission that there is any requirement/entitlement associated with this; all costs that have been incurred by Weinstein in relation to your hotel and pet boarding, at this point, are considered as an offset (to the extent that becomes necessary).

Please confirm receipt of this email; I am out tomorrow in a deposition most of the day, but again, will update as to The Worthington's ability to extend your reservation for your current room.

Thank you -

Glynis L. Zavarelli
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